

Quality Improvement Manual

A Practical Toolkit for Faith-Based Health Facilities



Christian Social Services Commission
First Edition, September 2025



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FOREWORD

As Tanzania's largest faith-based umbrella organization, the Christian Social Services Commission (CSSC) is steadfastly committed to excellence in service delivery across our network of over 965 health and 1,000 education facilities. We are proud to present this Quality Improvement (QI) Manual, a cornerstone resource designed to translate our commitment into consistent, measurable action. This guide serves as a vital bridge between national health goals and our grassroots service providers, ensuring that quality, equity, and compassion remain the hallmarks of our work.

In the journey towards universal health coverage, simply providing services is no longer enough. We must continuously innovate and improve. This manual is a practical roadmap for all our health facilities to build a culture of quality. It offers clear guidance on forming effective QI teams, designing programs to address service gaps, and implementing changes that result in measurable improvements such as reducing wait times, enhancing medicine availability, and improving patient communication.

This manual is tailored to empower leaders at every level of our health system. For Diocesan Health Secretaries, it is a compass for standardizing QI approaches, mentoring facility leaders, and aligning local initiatives with national health policies. For Health Facility Management Teams, it is a practical blueprint for integrating QI into daily operations, strategic planning, and budgeting. For our frontline QI Teams and healthcare workers, it is a source of empowerment, demonstrating how their direct contributions lead to better patient outcomes and stronger communities.

As a faith-based organization, our pursuit of quality is more than a technical requirement; it is a moral and spiritual calling. The communities we serve seek not only treatment but also dignity, compassion, and hope. This manual is therefore both a technical tool and a spiritual guide, reminding us that quality improvement is an act of stewardship a commitment to using our resources wisely and remaining accountable to those entrusted to our care.

By implementing this manual, the facilities can expect improved patient satisfaction and health outcomes; enhanced staff collaboration and morale; Stronger internal governance and accountability and greater resilience and adaptability in service provision. We call upon every leader, practitioner, and stakeholder to embrace this manual and make it an equitable living document that informs your daily work. Together, let us continue to demonstrate that faith-based facilities in Tanzania are models of quality, equity, and resilience.



Peter Maduki

Executive Director

Christian Social Services Commission (CSSC)

ACKNOWLEDGEMENT

On behalf of the Christian Social Services Commission (CSSC), I extend my deepest gratitude to all those who dedicated their time, expertise, and passion to the design and development of this Quality Improvement (QI) Manual. This document represents not only a technical achievement but also a shared commitment to advancing the quality of health care across the faith-based health facilities in Tanzania.

I wish to begin by sincerely recognizing the immense support we received from the US Department of state through the PATA project to fund the project since it was just an idea up to completion. Secondly, Dr. David Isaya, our Quality Improvement Coordinator, whose leadership and dedication in organizing the team made this work possible. Special thanks go to our committed QI Officers; Dr. Glory Nyagwa, Dr. Victor Mangowi, Dr. Ludovick Lopa, Ms. Upendo Mkolangunzi, Ms. Judith Mwerero, Mr. Scalion Miyonga, and Mr. Adam Machalila who carried out the actual task of drafting, refining, and ensuring that the manual is both practical and comprehensive.

We also acknowledge the invaluable contribution of Dr. Godfrey Kway, the PATA Project Manager, for his strategic guidance and for mobilizing the resources that enabled this process to be completed successfully. In addition, we are particularly grateful to our external collaborators: Dr Eliudi Eliakimu, Director of the Health Quality Assurance Unit, Mr. Saturini Manangwa, the Assistant Director Quality of Nursing and Midwifery Services and Dr Chrisogone German, head of the Supportive Supervision Sub-unit in the Health Quality Assurance Unit MOH, for their guidance in structuring and reviewing the material, also, Dr. Peter Risha, the Director of SafeCare, PharmAccess, Tanzania, for his thoughtful contributions that enriched the document.

This manual would not have been possible without the faith-based health facilities across the country, who generously shared their experiences, challenges, and best practices. Their insights ensured that this manual is not only theoretically sound but also grounded in real practice. While we cannot mention each facility by name, we value their contribution deeply. Finally, I extend my appreciation to the entire CSSC team, whose continuous support provided the foundation for this collective achievement.



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EXECUTIVE SUMMARY

In response to growing demand for consistent, patient centered, and accountable healthcare in Tanzania, the Christian Social Services Commission (CSSC), has developed this Quality Improvement (QI) Manual as a strategic resource to serve as a comprehensive reference document for faith-based health facilities. Currently, the faith-based health facilities are faced with challenges such as high staff turn-over rate, attrition of well-trained-qualified staff, professional burn-out, poor quality healthcare services, dilapidated infrastructure etc. However, there is no single, simplified document to guide these health facilities towards improvement of the existing conditions. This manual provides a structured framework to strengthen the provision of quality health services at all levels of care.

The primary objective of this manual is to guide a wide range of stakeholders including - facility owners, Diocesan Health Secretaries, Boards, Management teams, QI Units, and individual health practitioners in establishing, operationalizing, and sustaining QI structures within their facilities. It provides clear direction on how to summon teams, establish Quality Improvement departments or units, define roles and responsibilities, and integrate QI practices into day-to-day service delivery. By doing so, the manual ensures that quality improvement is not seen as an isolated activity but as an integral component of health care management and practice.

The manual introduces fundamental concepts of Quality Improvement and Quality Assurance, clarifying their meaning, scope, and interrelationship. It further defines the dimensions of quality such as effectiveness, efficiency, equity, safety, timeliness, and patient-centeredness offering health practitioners a common language and foundation upon which to build QI initiatives. In addition, it provides an overview of different QI approaches and methodologies, allowing facilities to select strategies most suitable to their contexts.

Another critical contribution of this manual is its explanation of the position and functions of QI structures at both the national and facility level. It outlines the roles of QI teams, Units, and directorates, emphasizing how they interact with broader governance and management systems. The manual also details the process of recruiting QI teams, including guidance on identifying appropriate candidates, designing job descriptions, and fostering a multidisciplinary approach that integrates clinical, administrative, and support staff.

To ensure practical application, the manual describes a set of essential QI activities that every health facility should undertake. These include regular performance assessments, data-driven decision-making, client feedback mechanisms, and structured QI projects targeting priority areas. It further highlights the importance of monitoring and evaluation in sustaining improvement, providing tools and indicators for measuring progress and outcomes of QI programs.

This manual is designed to be a living document one that health facilities can continually reference as they confront evolving challenges and opportunities. It not only equips facilities with technical guidance but also reinforces a culture of accountability, teamwork, and learning. Ultimately, its implementation will enable facilities to identify service gaps, design evidence-based interventions, and enhance patient outcomes, while ensuring that services remain affordable, equitable, and aligned with national health priorities.

In conclusion, this QI Manual aligns with the Tanzania's health strategy and supports the broader goal of the universal health coverage. The Manual provides a roadmap for excellence in faith-based health service delivery in Tanzania. Thus, faith-based health facilities are invited to adopt this manual as a cornerstone of their quality journey. CSSC will support implementation through training, mentorship and ongoing technical assistance. Together we can move beyond the compliance and towards excellence in health care delivery in Tanzania.

ABBREVIATIONS

ANC	Antenatal Care
BLS	Basic Life Support
CHMT	Council Health Management Team
CPR	Cardiopulmonary Resuscitation
CQI	Continuous Quality Improvement
CSSC	Christian Social Services Commission
CTC	Care and Treatment Clinic
FB-HF	Faith-Based Health Facility
FBO	Faith-Based Organization
HBB	Helping Babies Breathe
HF	Health Facility
HMIS	Health Management Information System
HMT	Hospital Management Team
HR	Human Resources
IPC	Infection Prevention and Control
IPD	Inpatient Department
ISO	International Organization for Standardization
ISS	Internal Supportive Supervision
KPI	Key Performance Indicator
MCH	Maternal and Child Health
MOH	Ministry of Health
MOI	Medical Officer In-Charge
MTUHA	Mfumo wa Taarifa za Uendeshaji Huduma za Afya (Health Management Information System in Tanzania)
OHS	Occupational Health and Safety
OPD	Outpatient Department
OSHA	Occupational Safety and Health Authority
PDSA	Plan-Do-Study-Act
QA	Quality Assurance
QI	Quality Improvement
QIP	Quality Improvement Plan

QIT	Quality Improvement Team
QPM	Quality Program Management
RCC	Respectful Compassionate Care
RCH	Reproductive and Child Health
RHMT	Regional Health Management Team
SBM-R	Standards-Based Management and Recognition
SOP	Standard Operating Procedure
SSI	Surgical Site Infection
TAT	Turnaround Time
TQIF	Tanzania Quality Improvement Framework
UHC	Universal Health Coverage
WHO	World Health Organization
WIT	Work Improvement Team

DEFINITION OF TERMS

- **Quality:** refers to “performance according to standards” or doing the right thing, the right way at the right time.
- **Quality Improvement Plan:** refers to a set of recommended tasks developed following a quality assessment to be followed in the improvement journey for a certain period of time.
- **Quality Control:** relates to compliance with pre-defined, measurable standards.
- **Quality Assurance:** is the process of verifying or determining whether products or services meet or exceed clients’ expectations and put measures to ensure quality is maintained.
- **Quality Improvement:** is a systematic effort to improve the quality of health system development and the delivery of health care services.
- **Continuous Quality Improvement:** is a progressive incremental improvement of processes, safety, and patient care. The goal may include improvement of operations, outcomes, systems processes, improved work environment, or regulatory compliance.
- **Total Quality Management:** It refers to an approach by which management and employees can become involved in the continuous improvement of the services aimed at embedding awareness of quality in all organizational processes.
- **Quality Improvement Team:** Is a group of selected, multilevel, multidisciplinary staff tasked to oversee improvement on the day-to-day performance in a health facility.
- **Work Improvement Teams:** are employees based small groups, which aim to provide every staff with opportunities for meaningful involvement and contribution in solving problems and challenges.
- **Quality Improvement File:** is repository that contains all quality related records.
- **Health Facility Board/Governing Committee:** is the facility's highest governing body, that provides strategic oversight of all operations. This is also a multi-disciplinary team representing the owner, community, staff and may include other people with special expertise.

CHAPTER 1: INTRODUCTION

Welcome! This manual is your go-to guide for improving the quality of care in our health facilities. Think of it as a pocket handbook or toolkit designed to help you, your team, and your facility provide the best possible healthcare every single day. The goal is simple: to give everyone from management to frontline providers a common understanding of Quality Improvement (QI). It breaks down what QI is, why it's crucial, and how you can fit it into your daily work.

This guide will help you and your team:

- Set up clear quality structures.
- Integrate quality checks into your daily routines.
- Review your work and find ways to get even better.

The development of this manual followed a consultative and evidence-based approach. It draws on national health policies, strategies, and guidelines issued by the Ministry of Health, as well as global frameworks such as SafeCare, ISQua, and ISO. Lessons from previous QI initiatives within faith-based facilities, documented best practices, and relevant research and case studies also informed its content. In addition, the manual incorporates field experiences, insights, and practical knowledge shared by CSSC teams, QI officers, and partner institutions. This combination of policy alignment, empirical evidence, and real-world practice ensures the manual is both practical and comprehensive

This manual provides a practical, step-by-step guide to establishing and sustaining Quality Improvement (QI) in faith-based health facilities. It covers the foundations of quality in healthcare, QI governance structures, capacity building, and implementation strategies, with a strong emphasis on monitoring and evaluation. Designed to align with national health priorities and global standards, the manual equips healthcare teams with tools, frameworks, and best practices to enhance patient outcomes, strengthen accountability, and build a culture of continuous improvement across all levels of care.

This manual is fully aligned with Tanzania's national health priorities, including the Universal Health Insurance (UHI) agenda and the pursuit of Universal Health Coverage (UHC). It translates national quality frameworks into practical, facility-level actions that strengthen governance, efficiency, and patient safety. In parallel, it integrates global quality standards such as, SafeCare, and ISO, ensuring that faith-based health facilities benchmark their services against internationally recognized best practices. By bridging local health priorities with global benchmarks, the manual positions facilities to deliver equitable, people-centered, and resilient healthcare that is both nationally relevant and globally competitive.

1.1 Rationale of the QI Manual

This manual was created to provide a practical, user-friendly guide for implementing QI in Faith-Based Health Facilities (FB-HFs). For years, various QI initiatives have been part of our network from CSSC's early supervision checklists and the 2002 "Indicators for Quality of Care" guide to the adoption of formal systems like 5S-Kaizen and SafeCare standards. These efforts have laid a strong foundation, but a need remained for a single, simple resource to guide harmonized daily practice.

While many comprehensive national and international quality guidelines exist, they can often be complex to apply at a facility level. This manual does not replace those official documents; instead, its purpose is to **simplify their implementation**. It translates high-level standards into actionable steps that can be integrated into the daily routines of all staff members.

Drawing on over 30 years of CSSC's direct experience and collaboration with public, private, and faith-based facilities, this manual offers a collection of proven best practices and valuable lessons learned. The ultimate goal is to **institutionalize a culture of quality**, providing healthcare teams with the clear structures, daily activities, and review processes needed to ensure every patient receives the best possible care.

1.2 Who should use the QI Manual

This manual is a practical tool for the different groups in the entire healthcare system as follows: **For frontline staff**, it's a hands-on guide. If you're facing a challenge, gather your colleagues, find the relevant chapter, and use the sample forms in the annex as a starting point. Feel free to adapt these tools for your own department they are yours to use! **For hospital management teams**, this manual provides a clear framework for building a culture of quality. Use it to guide the recruitment of your QI team, prioritize improvement projects, and effectively budget for these vital initiatives.

Ultimately, this manual is for the **patients and communities** we serve. By setting clear standards for high-quality care, we aim to empower you. This guide serves as our public commitment to excellence, helping you understand the level of safe, effective, and compassionate care you should expect and deserve from our facilities. When patients are informed, they become powerful partners in their own health and advocates for continuous improvement in their communities.

1.3 Organization of the QI Manual

This manual is organized into three distinct sections to guide readers through both foundational context and practical application. Part One offers essential front matter, including the table of contents, list of abbreviations, a foreword, acknowledgements, and an executive summary that sets the stage for the manual's purpose and scope.

Part Two comprises Chapters One through Five, presenting the core content ranging from conceptual frameworks to implementation strategies for quality improvement in faith-based health facilities.

Part Three features a comprehensive annex filled with practical resources: curated best practices, key performance indicators, sample forms, protocols, reference notes, and standard operating procedures (SOPs). These attachments are designed to support implementation and serve as ready-to-use tools aligned with the topics discussed in the main chapters.

1.4 Foundations of Quality Improvement

This section lays the groundwork for all your QI work. We'll start by defining what quality means in healthcare and then explore the key concepts, structures, and models you'll use to make a real difference in patient care.

1.4.1 Understanding Quality in Healthcare

Before we can improve quality, we must first understand what it is. At its heart, **Quality** means doing the right thing, for the right patient, at the right time to achieve the best possible results.¹ This means our health services must consistently provide care that is based on the best medical knowledge, protects people from preventable harm, and honours each individual's needs and preferences. Ultimately, quality is the degree to which our care consistently increases the likelihood of positive health outcomes for the people and communities we serve.

To systematically define and measure the quality of healthcare, the World Health Organization (WHO) identifies seven key elements, (Figure 1.1). These are also called dimensions of healthcare quality.



Figure 1.1 Elements of healthcare quality.

High-quality services are;

- *Effective*: deliver evidence-based care that improves health outcomes for individuals and populations.
- *Safe*: actively minimize the risk of harm and adverse events to patients during care delivery.
- *Timeliness*: reduce waits and potentially harmful delays for both recipients and providers.
- *People-centered*: responsive to individual patient preferences, needs, and values.
- *Efficient*: maximize the benefit of available resources while avoiding waste in equipment, supplies, and energy.
- *Equitable*: provide a consistent standard of care that does not vary in quality due to personal characteristics such as gender, ethnicity, or socioeconomic status.
- *Integrated*: provide coordinated and continuous care across different providers, departments, and levels of the health system to ensure a seamless patient experience.

1.4.2 Quality's Role in the Health System.

Strengthening health systems (Figure 1.2) is both a global and national priority, with a strong emphasis on embedding quality at every level of care. In Tanzania, the Ministry of Health leads this effort, guided by national strategies, guidelines, and QI frameworks. These initiatives aim to standardize care, from national hospitals to local dispensaries. By prioritizing QI, Tanzania's health system is working not only to expand access to services, but also to guarantee that every citizen receives a quality healthcare. This is a critical step toward achieving Universal Health Coverage (UHC) and better health outcomes for all. Integrating QI into daily practice among FBO facilities will foster a culture of trust, accountability, and continuous learning, while contributing to the overall strengthening of Tanzania's healthcare system. Studies have shown that more people are dying from poor quality services rather than lack of access to care (Kurk et al). Meaning that, Quality care helps in improving patient safety by minimising potential harm and preventable deaths. This is important since in UHC more people will be provided with access so it is important to address quality as part of UHC.

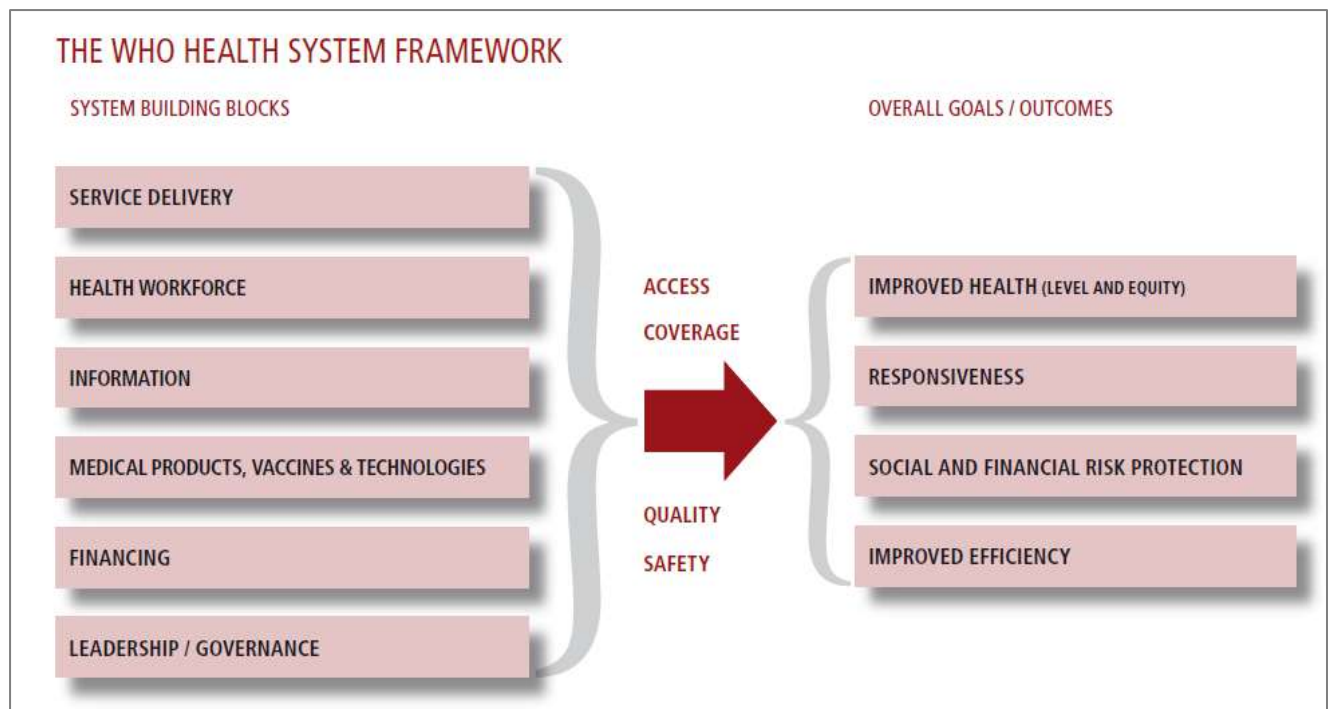


Figure 1.2 WHO Health System Building Blocks.

1.4.3 Practical Models for Improving Quality

Now that you've grasped the key concepts, it's time to move from theory to practice. The following two models are widely recognized and highly effective tools for implementing QI initiatives at your facility. They will help you structure your efforts, drive measurable change, and foster a culture of continuous learning and accountability.

A. Donabedian model

The Donabedian Model, developed by Dr. Avedis Donabedian, is a simple framework for measuring and improving healthcare quality. It breaks down the delivery of care into three distinct components: **Structure**, which includes the resources and setting where care occurs, such as facilities, equipment, and staff levels; **Process**, which covers the methods and procedures used in providing care, like conducting hygiene audits or training staff; and **Outcome**, which is the final effect on a patient's health status and satisfaction. The model's core principle is that a good Structure enables an effective Process, which in turn leads to a positive Outcome.

For example, having available hand washing facilities (Structure) supports consistent hand hygiene practices (Process), ultimately reducing hospital-acquired infections (Outcome). By analysing these three elements, health facilities can identify specific weaknesses in their system and implement targeted changes to improve the overall quality and safety of patient care.

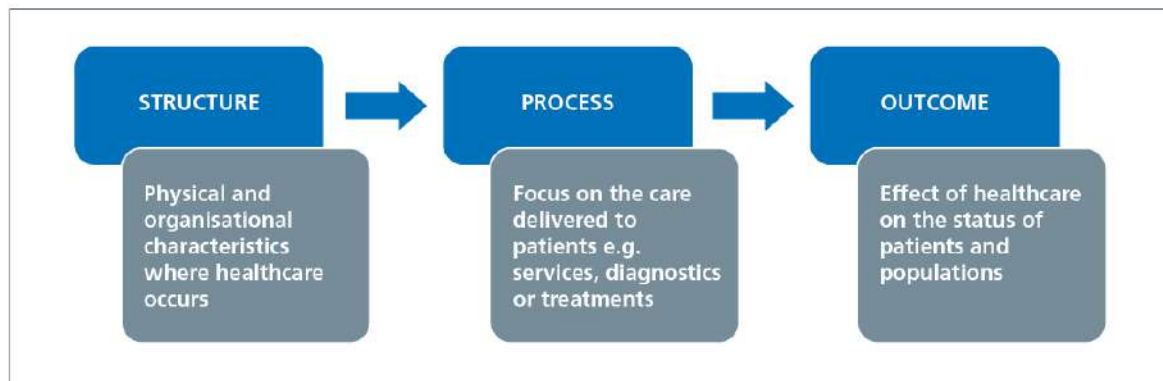


Figure 1.3 Donabedian Model.

B. Model for Improvement

The Model for Improvement is a structured framework that helps healthcare teams test and implement changes to enhance patient care. It begins by answering three fundamental questions to give the project a clear direction: "What are we trying to accomplish?" which sets the **Aim**; "How will we know that a change is an improvement?" which establishes the **Measures** for success; and "What changes can we make that will result in an improvement?" which generates specific **Change Ideas**. This first step ensures that any improvement effort is focused and measurable from the start.

Once a change idea is chosen, the model uses the Plan-Do-Study-Act (PDSA) cycle to test it on a small scale (Figure 1.4). This four-step process involves planning the test (Plan), carrying it out (Do), observing and analysing the results (Study), and determining what to do next (Act). The PDSA cycle is meant to be repeated in rapid, iterative loops, allowing teams to learn from each test, refine their approach, and build confidence that a change will work before implementing it more broadly.

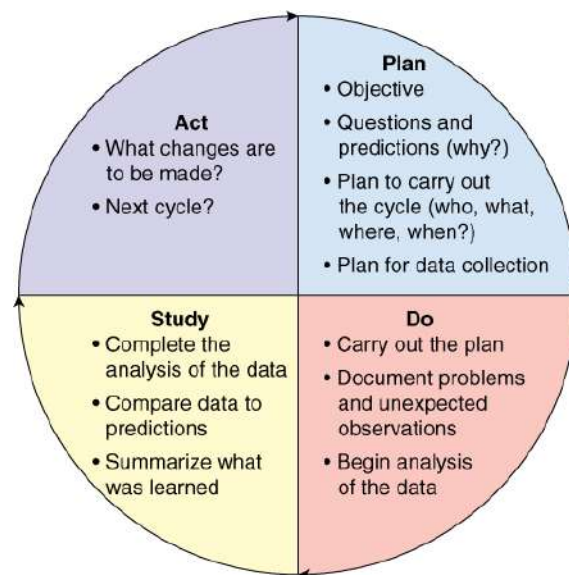
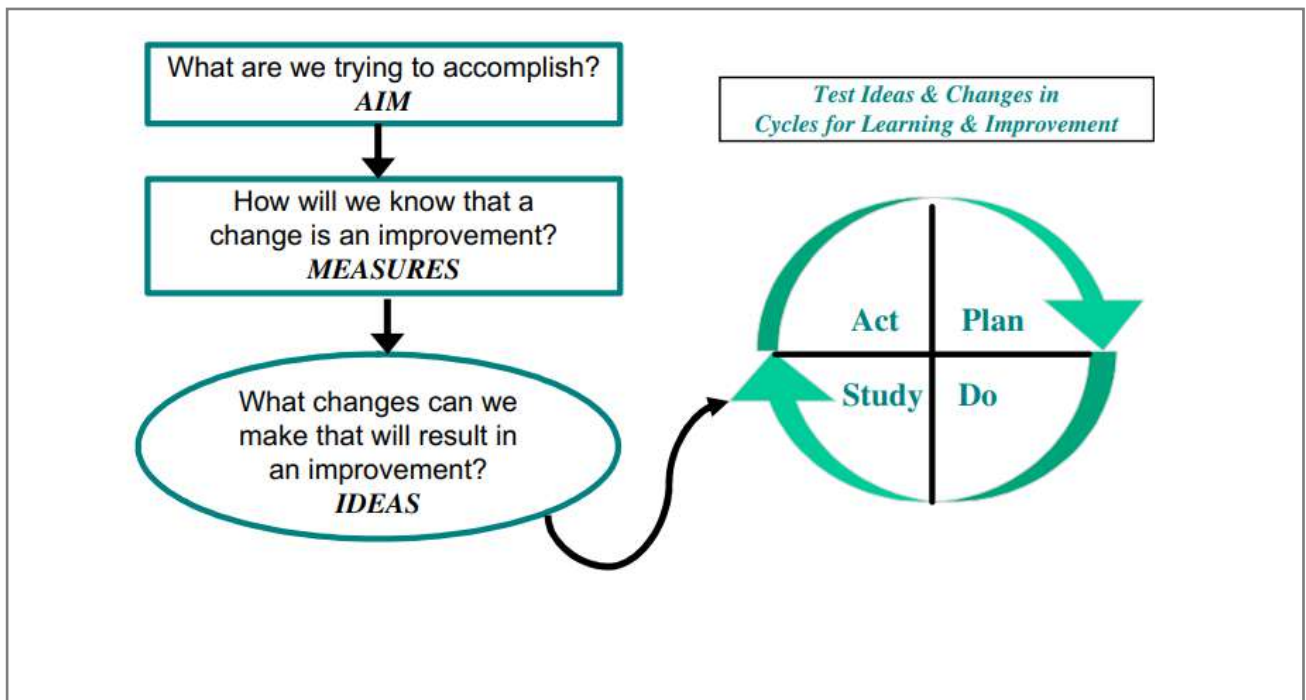


Figure 1.4 Model for Improvement.

CHAPTER 2: QI STRUCTURES AND GOVERNANCE

This chapter introduces the essential frameworks of QI within a health facility, focusing on its structures, governance, and management. It explores the critical link between these organizational components and the successful implementation of QI interventions. The effectiveness of any quality initiative is profoundly influenced by its leadership, a principle encapsulated in John C. Maxwell's famous adage, "Everything rises and falls on leadership." Consequently, the success of QI hinges on the commitment, understanding, and attitude of leaders toward quality enhancement.

To provide a comprehensive overview, this chapter systematically outlines the specific roles and functions at each level of the healthcare system, from the facility's management down to the community where services are ultimately delivered and consumed.

2.1 Health Facility Board/Governing Committee

As the facility's highest governing body, the health facility board/Governing Committee provides strategic oversight of all operations. Its responsibilities include advising management on performance enhancement and approving key decisions. To effectively guide quality efforts, the board must have a solid understanding of QI interventions. This knowledge enables them to set specific performance indicators for the management team, which must then be reported back to the board as a substantive agenda item in every meeting to ensure accountability.

2.1.1 Composition of the Board

The composition of a health facility's board or governing committee varies depending on the facility's size and structure. However, it is standard practice to include community representatives who act as the voice of the clients, ensuring that perspectives on quality care are considered. Furthermore, the specific criteria for board membership and the selection process are typically defined by the health facility's own policies and governance guidelines.

For FBO facilities, CSSC provides two foundational documents for guidance. Health facilities are strongly encouraged to adopt and use these two documents as key references alongside this manual.

- *CSSC Board Charter*: Outlines the procedures for establishing and running an effective hospital board.
- *CSSC Hospital Operational Manual*: Details how the management team can best handle daily hospital operations.

2.1.2 Roles and Responsibilities of the Board

The following are the roles of the health facility board in ensuring effective implementation of the quality improvement activities in the facility:

- Setting the facility's strategic direction for quality improvement, using the strategic plan to guide management priorities.
- Securing financing for quality improvement interventions.
- Approving budgets allocated for Quality Improvement (QI) initiatives.
- Representing community feedback on the health facility's operations.
- Establishing Quality Improvement as a standing agenda item in all board meetings.
- Reviewing and advising on QI activity reports submitted by the management team.
- Overseeing patient safety through monitoring compliance with WHO recommended patient safety priority areas by creating systems for monitoring performance for example using the ISQua accredited healthcare standards such as SafeCare standards. (Clinical Governance)
- Organizing external performance evaluations using ISQua approved standards and methodology e.g. SafeCare standards
- Exercising resource stewardship by allocating funds for QI and monitoring their efficient use, including requiring a set QI budget percentage before approving the facility's annual budget.
- Holding the HMT accountable to quality health service delivery based on priority outcome/output indicators aligned to patient safety priority areas.

2.2 Health Management Team

The Health Management Team (HMT) is the executive body responsible for overseeing the daily operations of the facility to ensure the consistent delivery of high-quality health services. Led by the Medical Officer-in-Charge (MOI)/Hospital director, the HMT is composed of the heads of all key departments and units. To embed quality assurance into the core of facility leadership, the chairperson of the QIT serves as an ex-officio member of the HMT. This automatic inclusion guarantees that quality improvement is a central component of all operational planning and decision-making.

2.2.1 Composition of the HMT

The composition of the HMT is tailored to the specific structure and level of the health facility. As a general rule, the team is composed of the heads of all major departments, both technical (e.g., clinical services) and supportive (e.g., administration, finance) who meet prescribed criteria for their roles, including their level of education. A foundational requirement for every HMT is the direct inclusion of the facility's quality improvement unit leadership. As will be detailed later, the title of this QI body varies by the facility's level:

- Zonal hospitals have a QI Directorate.
- Regional referral hospitals have a QI Unit.
- Primary healthcare facilities have a QI Team.

Regardless of the title, the leader of that quality structure is an automatic member of the HMT. This integration is designed to streamline the flow of information in both directions, ensuring that management decisions are informed by quality data and that quality initiatives are fully supported by leadership.

2.2.2 Roles and Responsibilities of the HMT

The following are the roles of the HMT in ensuring effective implementation of the quality improvement activities in the facility:

A. Strategic Planning & Direction

- Developing the facility's strategic and business plans in line with the board's guidance.
- Formulating the organization's vision and mission statements specifically for Quality Improvement (QI).
- Disseminating the strategic plan and QI vision to all staff to ensure alignment.

B. QI Structure & Oversight

- Establishing and maintaining a functional QI structure and dedicated teams.
- Overseeing all QI processes and the functions of the Quality Improvement Team (QIT).
- Benchmarking selects service delivery outcome /output indicators to prioritize improvement areas.

C. Resource Management

- Financing QI initiatives by approving budgets and ensuring the timely disbursement of funds.
- Maintaining a reliable supply chain for the equipment and supplies essential for delivering quality care.

D. Performance & Accountability

- Reviewing and acting upon QI reports submitted by the QIT during management meetings.
- Conducting daily internal supervision to ensure staff adhere to quality standards.
- Using data for evidence-based planning and continuous facility improvement.
- Instituting facility wide Quality Culture (quality is responsibility of every one and not of the QIT).

2.3 Quality Improvement Team

The Quality Improvement Team (QIT) serves as the custodian of all QI initiatives within the facility, ensuring that efforts to enhance care are coordinated, inclusive, and sustainable. To reflect the diversity of services and perspectives across the facility, the team is encouraged to be multidisciplinary by bringing together representatives from clinical, administrative, support, and leadership roles. This inclusive approach fosters ownership, collaboration, and relevance in the changes being pursued.

The QIT plays a strategic role in identifying gaps in service delivery, systems, or processes that require improvement. Once priority areas are defined, the team develops targeted strategies, implements interventions, and monitors progress using agreed-upon indicators. Their work is guided by national QI frameworks and facility-specific goals, with a focus on achieving measurable improvements in safety, efficiency, and patient experience. Through regular review and adaptive learning, the QIT helps embed a culture of continuous improvement and accountability across all levels of the organization. Figure 2.1 highlights the key qualities of QIT members.

2.3.1 Composition of the QIT

The QIT's composition is designed to be inclusive, incorporating diverse perspectives from across the facility.

- **Chairperson:** serves as the lead of the QIT providing strategic direction and facilitating team activities. This individual may be appointed by the facility's MOI or elected by QIT members through a consensus-based process. The Chairperson is responsible for guiding discussions, ensuring alignment with facility goals, and fostering collaboration across departments to drive meaningful quality improvement.
- **Secretary:** The Chairperson may appoint a secretary from among the members. This person's role is to schedule and facilitate meetings, record minutes, and track the implementation of action plans.
- **Multidisciplinary Members.** The team should consist of a diverse mix of staff from various clinical departments and administrative units. This typically includes doctors, nurses, pharmacists, laboratory experts, and other personnel who have experience with or a passion for quality improvement. The number of members is flexible, usually ranging from 5 to 15 people. The ideal size depends on the level of the health facility and the complexity of its services.

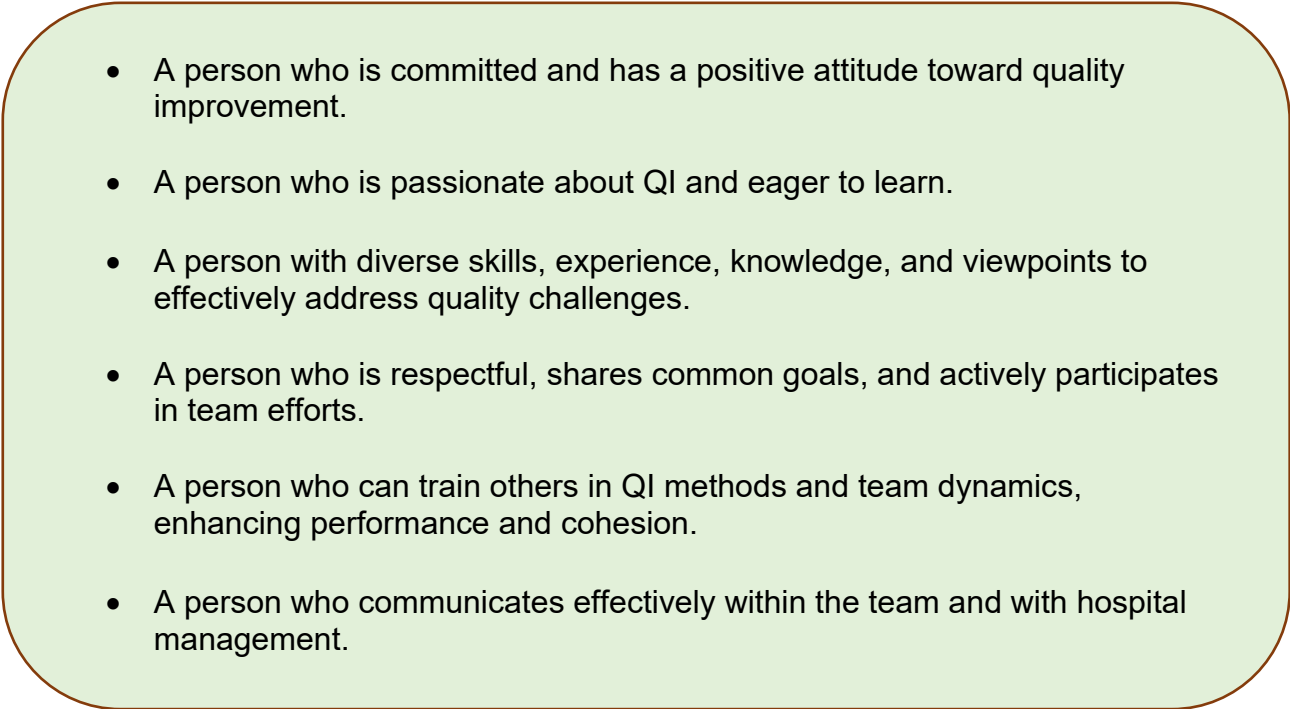
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- A person who is committed and has a positive attitude toward quality improvement.
 - A person who is passionate about QI and eager to learn.
 - A person with diverse skills, experience, knowledge, and viewpoints to effectively address quality challenges.
 - A person who is respectful, shares common goals, and actively participates in team efforts.
 - A person who can train others in QI methods and team dynamics, enhancing performance and cohesion.
 - A person who communicates effectively within the team and with hospital management.

Figure 2.1 Essential qualities of an effective QIT member

- Provides a clear roadmap and systematic approach for all QI projects.
- Sets and communicates priorities to focus the team's efforts.
- Supports team members by offering recognition and encouragement.
- Fosters an environment of innovation, creativity, and learning.
- Organizes and leads regular, productive QIT meetings with the secretary.
- Liaises with external health organizations (like the MOH, CSSC) to stay current with best practices.
- Reports on QI progress to the facility's management team.
- Advocates for the necessary resources (budget, time, etc) to ensure QI plans succeed.

Figure 2.2 Roles and responsibilities of QIT chairperson

- Develop and manage the annual calendar for all QI activities.
- Compile the annual Key Performance Indicators (KPIs) for monitoring.
- Record, document, and archive minutes from all quality-related meetings.
- Serve as the official custodian for all QIT documents and records.
- Ensure all QI reports are prepared and submitted on schedule.
- Manage the orientation program for new QIT members.
- Assist in coordinating and orienting subcommittee members (e.g., IPC, Risk).
- Liaise with departmental teams (like WITs) to support quality implementation.
- Act as the team's training coordinator.
- Work with HR and senior leaders to orient all facility staff on their quality-related duties.

Figure 2.3 Roles and responsibilities of QIT secretary

2.3.2 Roles and Responsibilities of the QIT

The Quality Improvement Team (QIT) drives a health facility's commitment to excellence, patient safety, and continuous learning. Its mandate is both strategic and operational in bridging the gap between high-level goals and the realities of day-to-day service delivery. To achieve this, the team undertakes a diverse set of responsibilities, encompassing everything from strategic planning and data analysis to staff training and enhancing the patient experience. These core functions are illustrated henceforth;

A. Planning & Strategic Implementation

- Developing and implementing the facility's annual QI plan in alignment with strategic goals.
- Identifying challenges and gaps in service delivery, conducting root cause analysis, and creating targeted action plans.
- Applying QI methodologies, such as the Plan-Do-Study-Act (PDSA) cycle, to guide improvement projects.
- Guiding staff adherence to national quality standards, clinical guidelines, and best practices.

B. Monitoring, Data Analysis & Assessment

- Monitoring and analysing key facility trends and performance indicators (e.g., patient turnover, Turn-Around Time (TAT), revenue vs. attendance).
- Conducting and overseeing various assessments and audits, including clinical audits, Star Rating, SBM-R, and Infection Prevention and Control (IPC) assessments.
- Performing daily monitoring of quality practices across all departments to ensure standards are consistently met.

C. Training & Capacity Building

- Conducting ongoing QI training and capacity building for all levels of staff, including the Health Management Team (HMT) and board members.
- Extending capacity building efforts to clients and the wider community to promote health literacy and engagement.

D. Reporting & Communication

- Producing and presenting regular progress reports on QI activities (daily, weekly, monthly, quarterly) for various forums, including morning huddles, HMT meetings, and board meetings.
- Facilitating regular monthly QIT meetings to review progress, discuss challenges, and plan next steps.

E. Patient Experience & Customer Care

- Overseeing all customer care operations, including the management of the patient feedback and complaints system.
- Analysing client satisfaction data to identify areas for improving the patient experience.

F. Team Coordination & Collaboration

- Coordinating the design and implementation of all improvement activities across the facility.
- Actively participating in related facility-wide committees and activities, such as IPC, Risk Management, and Occupational Health and Safety (OHS).

2.4 Work Improvement Team

Work Improvement Teams (WITs) are small, specialized groups formed within individual departments or sections of a health facility. Composed of staff who work together in the same area, these teams serve as frontline drivers of quality in translating facility-wide strategies into practical, department-specific actions. In departments such as the Care and Treatment Clinic (CTC) and Reproductive and Child Health (RCH), WITs may include Community Health Workers (CHWs), who play a crucial role in gathering and relaying perceptions, complaints, and suggestions from the community, while also promoting health and facilitating timely referrals. Importantly, WITs form a vital link between the central QIT and the day-to-day work at the unit level, ensuring that quality initiatives are relevant, effectively implemented, and sustained over time.

Roles and Responsibilities of the WIT

- Participating in regular departmental QI meetings to discuss progress and challenges.
- Implementing quality improvement activities and directives cascaded down from the main facility QIT.
- Contributing ideas, feedback, and frontline insights from the department back to the central QIT.
- Leading problem-solving initiatives to identify and resolve quality gaps specific to their own department.
- Ensuring that departmental staff receive and effectively apply relevant QI training.
- Monitoring daily quality practices within the department, such as using data to measure adherence to the WHO Patient Safety Priority areas.

2.5 Community and Clients

Engaging the community is a cornerstone of a responsive and accountable healthcare system. When clients and community members are treated as active partners rather than passive recipients of care, they play a vital role in driving continuous quality improvement. Their involvement ensures that services are people-centered, transparent, and tailored to meet the population's actual needs. The community's key roles in this process are outlined below.

Roles and Responsibilities of the Community and Clients

- Demanding high-quality care by knowing their rights as patients.
- Providing feedback through suggestion boxes, surveys, and interviews.
- Participating in the planning of health services to meet community needs.
- Helping to identify and address barriers to care for all populations.
- Acting as public advocates and champions for quality healthcare.
- Helping to mobilize community resources to support QI initiatives.

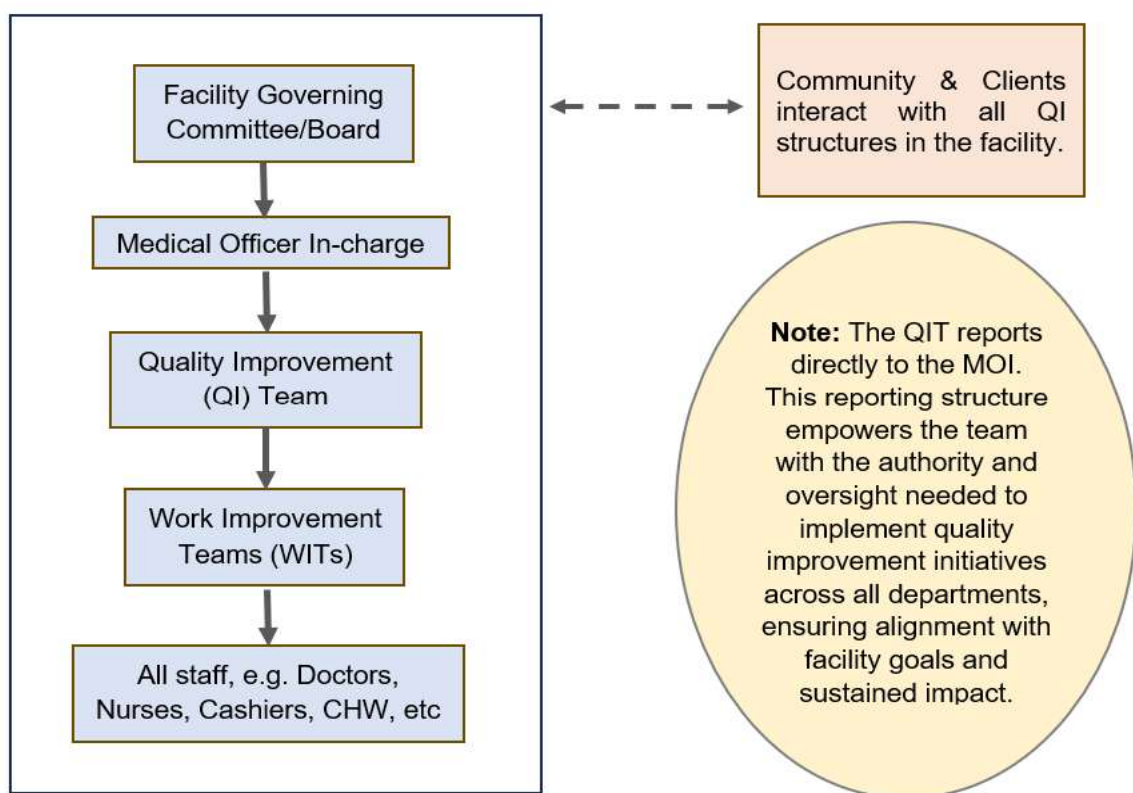


Figure 2.4 Quality Improvement structures at a health facility.

Table 2.1 Quality Improvement structures; roles and responsibilities.

Quality Structure	Roles and responsibilities
Facility Governing Committee/ Board	• Define quality vision and approve quality related policies. • Track performance using quality indicators.
Facility Management Team	• Turn strategic quality policies into operational plans and ensure facility-wide implementation. • Monitor performance data and identify systemic issues need solving.
Quality Improvement (QI) Team	• Leading QI Initiatives. • Capacity Building & Mentorship to staff on QI tools and techniques.
Work Improvement Teams (WITs)	• Frontline Problem Identification. • Implementing Local Solutions.
Healthcare Workers	• Adherence to Clinical Standards. • Accurate Documentation & Feedback.
Community Health Workers (CHWs)	• Collect and relaying perceptions, complaints, and suggestions. • Health Promotion & Referrals.
Community and Clients	• Actively participate in feedback mechanisms (e.g., suggestion boxes, satisfaction surveys). • Engage with health education, adhere to treatment plans, and advocate for high-quality, respectful, and accessible health services.

CHAPTER 3: TRAINING AND CAPACITY BUILDING

An effective Quality Improvement (QI) program is built on the knowledge and skills of its people. Continuous training and capacity building are not optional activities; they are the foundation for creating a culture of excellence and patient safety. To successfully implement and sustain QI, it is essential that every individual from frontline staff and community members to the governing board is empowered with the right knowledge for their specific role.

This chapter outlines the need for continuous QI training, a structured approach to training, detailing who needs to be trained, what they need to learn, and how this training can be effectively delivered. A well-executed training plan ensures that QI practices are understood, adopted, and consistently applied, leading to measurable improvements in patient outcomes and facility performance.

3.1 Need for Continuous QI training

Continuous training is the backbone of successful quality improvement. It equips healthcare teams with the skills, mindset, and confidence needed to drive change, adapt to evolving standards, and deliver safe, effective, and compassionate care across all levels of the health system. Here are the key reasons why continuous training is important for effective QI initiatives;

- **Provides Essential Skills:** It equips QIT with the necessary knowledge, skills, and tools to implement effective quality improvement practices.
- **Fosters a Culture of Improvement:** It creates an environment of continuous learning, enabling teams to consistently identify problems, streamline processes, and enhance patient outcomes.
- **Keeps Staff Updated:** It helps staff stay current with the latest methodologies, best practices, and evidence-based approaches, leading to sustained improvements in quality and safety.
- **Builds Capacity and Resilience:** Training strengthens the ability of staff to manage complex situations, lead change, and maintain improvement efforts over time.
- **Increases Engagement and Morale:** It encourages staff to take ownership of quality initiatives, which improves their motivation and overall morale.

- **Empowers Teams:** By establishing quality improvement as a core competency, it helps develop empowered teams that can make meaningful and lasting changes.
- **Improves Data and Technology Use:** It aids staff in applying data-driven decision-making and using technology effectively to drive improvements.
- **Leads to Better Outcomes:** The ultimate result of ongoing education is improved patient safety, reduced medical errors, enhanced efficiency, and higher satisfaction for both patients and providers.

3.2 A Targeted Approach to QI Training

To ensure continuous training is relevant and efficient, we recommend a “targeted approach.” This model delivers content tailored to the specific roles and responsibilities of each group in the quality structures of the health facilities.

3.2.1 QI Leadership & Governance Training

This training is mainly useful for facility boards and HMTs. For leadership, training focuses on high-level oversight, strategic direction, and resource allocation. The goal is to secure their buy-in and active support for all QI initiatives.

Key Training Topics:

- **The Business Case for Quality:** Understanding the link between quality improvement and the facility's financial health and reputation.
- **Governance for Quality:** The specific roles and responsibilities of the board and HMT in driving the QI agenda.
- **Strategic Oversight:** How to set meaningful QI indicators, interpret performance reports, and hold the facility accountable for results.
- **Fostering a QI Culture:** Techniques for leading change and embedding quality as a core organizational value.
- **Sustainability Planning:** Mechanisms for ensuring that QI programs are adequately financed and sustained over the long term.
- Patient Safety focused service delivery.

3.2.2 Technical Leadership Training

This training targets QIT. As the technical backbone of the QI program, the QIT requires comprehensive, in-depth training in QI methodologies and management, as recognized by the Ministry of Health and advocated by CSSC.

Key Training Topics:

- **Advanced QI Models:** Deep understanding of methodologies like Plan-Do-Study-Act (PDSA) and Donabedian models.
- **Data Management & Analysis:** Skills in data collection, analysis, and using data to drive decision-making (e.g., Clinical Audits, Client Satisfaction Surveys).
- **Complex Assessments:** Competency in conducting and interpreting various assessments (e.g., Star Rating, SafeCare).
- **Patient Safety & Risk Management:** Strategies for managing patient safety, implementing antimicrobial stewardship, and reducing medical errors.
- **Training & Mentorship:** How to train and coach other staff members, including WITs and healthcare providers.

3.2.3 Training for WIT & Healthcare Providers

This group consists of the "first-line champions" who implement QI during routine service delivery. Their training must be practical, hands-on, and directly applicable to their daily work.

Key Training Topics:

- **QI Fundamentals:** Basic concepts of quality improvement and their specific roles in the process.
- **Adherence to Standards:** The importance of following Standard Operating Procedures (SOPs), clinical guidelines, and algorithms.
- **Patient-Centered Care:** Principles of respectful and compassionate care, customer service, and effective communication.
- **Practical QI Tools:** Introduction to basic quality tools and participation in assessments like Clinical Audits and supportive supervision.
- **Teamwork & Problem-Solving:** How to work effectively within their WITs to identify and solve department-level quality issues.

3.2.4 Community & Client Engagement

Empowering patients and communities make them active partners in quality improvement. Orientation on rights, responsibilities, and feedback channels builds trust, strengthens referrals, and enhances care. Their involvement ensures services are responsive, compassionate, and effective in driving sustained improvements and reinforcing the mission of people-centered healthcare in every facility.

Key Training Topics:

- **Patient Rights & Responsibilities:** Understanding the Client and Service Charter.
- **The Role of Feedback:** The importance of participating in surveys, using suggestion boxes, and providing constructive feedback.
- **Community's Role in Quality:** How their active participation helps ensure accountability and people-centered care.

3.3 Training Modes

To maximize impact, the facility should use a blended approach to training delivery. This strategy combines various methods such as traditional classroom sessions, practical on-the-job coaching, and interactive e-learning to cater to diverse learning styles. By reinforcing knowledge in multiple ways, this approach boosts engagement, improves retention, and ensures skills are effectively applied.

- A. On-the-Job Training & Mentorship.** This is a practical, real-time coaching to embed skills into daily practice. Example: a senior nurse observes and guides a new nurse on correct sterile wound dressing techniques in the real ward setting.
- B. Classroom & Virtual Learning.** Structured sessions for foundational knowledge (can be in-person or online). Example: the hospital pharmacist conducts a scheduled lecture for all clinical staff on the new national antimicrobial stewardship guidelines.
- C. Simulation Exercises:** Hands-on learning for emergency scenarios (e.g., fire drills, CPR). Example: The team resuscitates a newborn mannequin that isn't breathing, practicing the critical "Golden Minute" protocol immediately after a simulated birth.

- D. **Team-Based Learning:** Collaborative sessions that build teamwork and communication. Example: The maternity Work Improvement Team collaborates to analyse a case study on reducing delays for emergency caesarean sections.
- E. **QI Projects (Action Learning):** Applying learned concepts by participating in or leading a real QI project. Example: A team implements a new surgical safety checklist to reduce surgical site infection over a three-month period.
- F. **External Opportunities:** Attending seminars and trainings from partners like CSSC. Trips to model hospitals to observe best practices firsthand. Example: A QIT member attends a regional seminar hosted by the Ministry of Health on new standards for patient-centered care.
- G. **Peer-to-peer learning:** Healthcare workers from one health facility may be organized and visit another HFs to learn and implement what works for them.

Table 3.1 Sample QI Annual Training Schedule

S/n	Topic	Sample date*	Responsible Person/Dept.
1	Introduction to SafeCare Standards	10/01/2025	QIT
2	Risk Management and Use of Risk Registers	18/01/2025	QIT / HMT
3	Clinical Audits: Methodology and Practice	25/01/2025	QIT
4	Teamwork and Leadership for Quality	04/02/2025	HR / HMT
5	Waste Management (IPC)	13/02/2025	IPC Committee
6	Fire Safety Drill and Evacuation	27/02/2025	OHS Committee
7	Cardiopulmonary Resuscitation (CPR) Refresher	11/03/2025	Clinical Dept.
8	Triage Principles and Practice	28/05/2025	Emergency Dept.
9	Management of Anaemia	11/06/2025	Clinical Dept.
10	Updates on Malaria Treatment Guidelines	24/06/2025	Clinical Dept.
11	Management of Shock	04/07/2025	Clinical Dept.
12	Decontamination & Sterilization Processes (IPC)	29/07/2025	IPC Committee
13	Management of Pre-Eclampsia	05/08/2025	MCH Dept.

NOTE: A comprehensive annual training plan is essential for fostering a robust QI culture. To ensure your program is effective, the schedule should incorporate these essential topics:

- **QI Methodologies :** PDSA, Root Cause Analyses, SafeCare Standards, etc.
- **Patient Safety:** Error reduction techniques, risk management, medication safety.
- **Data for Improvement:** Clinical audits, satisfaction surveys, use of data tools.
- **Clinical Excellence:** Evidence-based practices, triage, top 10 diseases.
- **IPC:** Waste management, decontamination, PPE use.
- **Emergency Preparedness:** Fire safety drills, resuscitation (BLS/ACLS).
- **Soft Skills:** Teamwork, leadership, communication, patient-centered care.
- **Facility Policies:** Updates on new or revised guidelines, SOPs, and policies.

CHAPTER 4: A STEP-BY-STEP GUIDE TO QI IMPLEMENTATION

Welcome to the implementation phase of your quality journey. This chapter is your hands-on guide to turning quality improvement plans into tangible results. Effective implementation is a systematic process of identifying gaps, understanding their root causes, testing changes, and monitoring progress to ensure lasting success.

To make this process clear and manageable, we will walk through a simple five-steps. By following these steps, your QIT and WITs can move from identifying a problem to embedding a sustainable solution into the facility's daily operations. The steps are outlined below;

1. Identify Areas for Improvement (Where are the gaps?)
2. Analyse the Root Cause (Why do the gaps exist?)
3. Develop a Quality Improvement Plan (QIP) (How will we fix it?)
4. Test & Implement Changes (Let's try our solution.)
5. Monitor, Sustain, and Spread Success (Did it work and can we keep it going?)

4.1 Identify Areas for Improvement

The first step is to find out where your facility is performing well and where there are gaps between your current practice and the set standards. This is done through assessments. Assessments are structured reviews that benchmark your services against established quality standards. Your QIT is responsible for organizing these assessments, whether they are internal (self-assessments) or conducted by external bodies (for example, the Ministry of Health or CSSC).

Your facility can leverage a range of structured tools to identify performance gaps and improvement opportunities. Each tool offers a distinct perspective, whether through data analysis, process mapping, or stakeholder feedback in helping teams uncover root causes and prioritize actionable, context-specific quality interventions. These tools are outlined next;

- **SafeCare Assessments:** An internationally recognized, stepwise approach that grades facilities from Level 1 to 5. CSSC advocates for this approach because it is both thorough and interactive. It provides a clear, motivating, and internationally recognized pathway for facilities to improve.

- **Star Rating Assessments:** A national tool for primary health facilities that assigns a rating from zero to five stars based on performance across key service areas.
- **Clinical Audits:** A focused review of patient care processes and outcomes to see if they align with best practices and standards. Clinical audits are organized by the facility internally using MOH's National guidelines and tools to guide the process.
- **Supportive Supervision:** A collaborative process where supervisors visit departments to provide mentorship and identify areas for improvement together.

4.2 Analyze the Root Cause

Once an assessment has identified a gap ("the what"), the next crucial step is to understand "the why". Simply treating the symptom won't create lasting change. Root Cause Analysis (RCA) is a set of problem-solving methods used to dig deeper and find the underlying cause of a problem. This section introduces two simple yet powerful Root Cause Analysis (RCA) tools for understanding quality gaps: **the 5 Whys** and **the Fishbone Diagram**.

A. The 5 WHYs

The 5 Whys is a simple technique used to find the root cause of a problem by repeatedly asking "Why?". It helps you look past the obvious symptoms to find the underlying issue.⁸

- You start with the problem and ask, "Why did this happen?" You then take that answer and ask, "Why did that happen?" You continue this process, with each answer becoming the basis for the next question, like peeling back the layers of an onion.
- While the name is "5 Whys," you might need more or fewer questions. You know you've found the root cause when the answer points to a broken process or system that, if you fix it, will stop the problem from happening again.

The "5 Whys" helps a team move beyond obvious, superficial answers (the symptoms) to find the deeper cause. Think of it like a doctor diagnosing an illness. They don't just treat a fever (the symptom); they ask questions to find the underlying infection (the root cause). Fixing the root cause provides a sustainable solution.

Problem Statement: A pregnant woman booked her first ANC visit in her third trimester, 30 weeks.

1. **WHY?** She said she didn't know she was supposed to come so early in the pregnancy.
2. **WHY?** There was no effective health education about the importance of early ANC in her village.
3. **WHY?** The local Community Health Worker (CHW) who provides this education has not visited her area in over six months.
4. **WHY?** The CHW's only means of transport (a bicycle) is broken, and there is no budget allocated for its repair.
5. **WHY?** The facility's community outreach budget is consistently prioritized for vaccination campaigns over maternal health education.

Root Cause: The analysis reveals the root cause is not simply the woman's lack of knowledge, but a systemic issue of resource allocation and strategic priority at the facility level that underfunds maternal health outreach.

Figure 4.1 How to use "5 WHYS"

B. The Fishbone (Ishikawa) Diagram

The **Fishbone Diagram**, also known as the **Ishikawa Diagram**, is a visual brainstorming tool used to explore all the potential causes of a specific problem. It gets its name because the finished diagram looks like the skeleton of a fish. The problem itself is written at the "head" of the fish. The main categories of potential causes are then drawn as "bones" branching off the spine, helping a team to organize their thoughts and systematically investigate the root of the issue. Let's explore our previous example to see how a fishbone would look like.

Problem Statement: A pregnant woman booked her first ANC visit in her third trimester, 30 weeks.

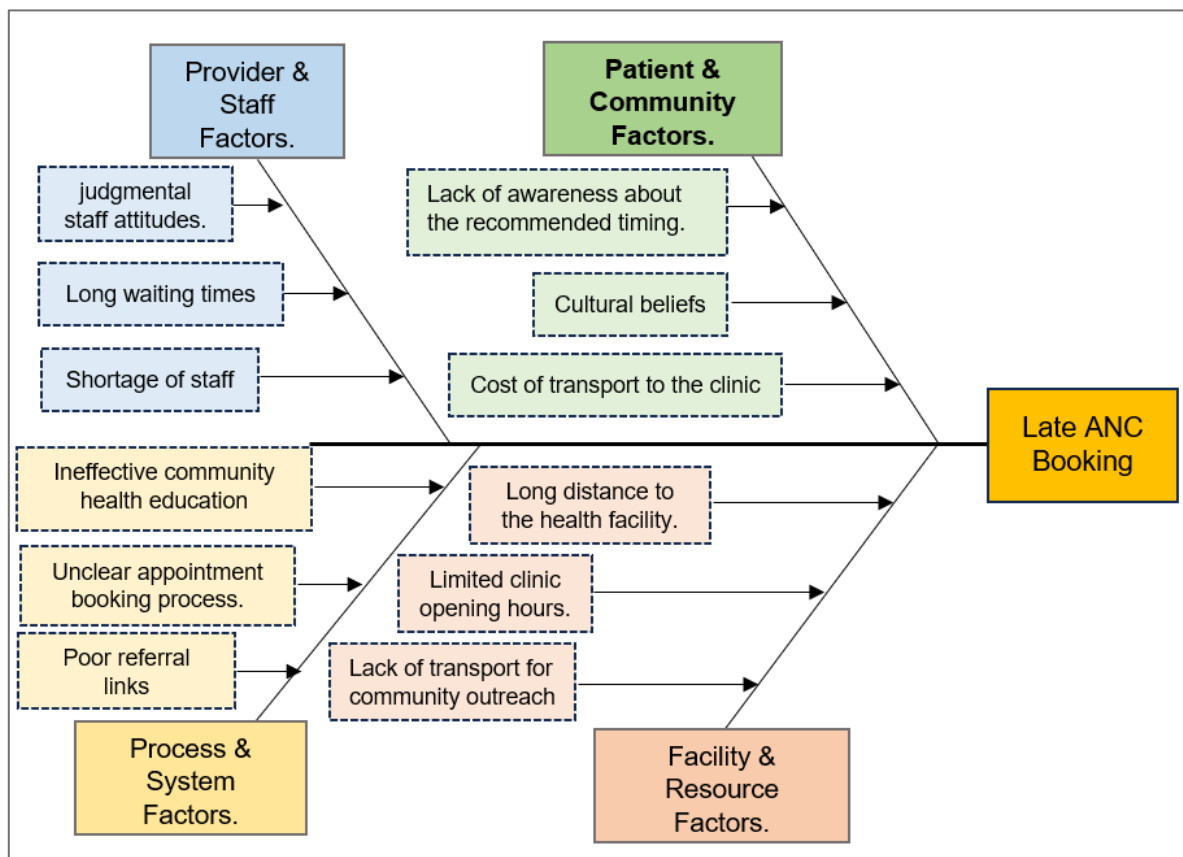


Figure 4.2 Fishbone diagram; root cause analysis for late ANC booking

Conclusion: The Fishbone Diagram shows that late ANC booking is a complex problem with many contributing factors. It helps the team see that the issue is not just about one thing, but a combination of patient, provider, resource, and system-level challenges that must be addressed.

4.3 Developing the Quality Improvement Plan

With a clear understanding of the problem and its root cause, it's time to create a formal plan of action. The Quality Improvement Plan (QIP) is your roadmap for change. It is a written document that turns your findings into specific, measurable, and time-bound tasks (Table 4.1). The QIT is responsible for developing the QIP, assigning tasks to relevant departments or individuals, and monitoring its implementation.

A well-structured QIP is critical because it:

- Focuses efforts on priority areas.
- Establishes clear goals and accountability.
- Tracks progress and ensure follow-through.

4.4 Testing & Implementing Changes

Instead of launching a large-scale, untested solution, QI relies on small, rapid tests of change. The most effective tool for this is the Plan-Do-Study-Act (PDSA) cycle. This four-stage cycle allows you to test an idea on a small scale, learn from the results, and refine it before wider implementation.

The Plan-Do-Study-Act (PDSA) cycle, which was fully explained in Chapter 1, provides a structured method for testing changes on a small scale before implementing them widely. It dr many different types of quality improvement initiatives. (A practical example is also available in Annex 1.1).

Below are a few examples of how this cycle can be applied in practice:

- **5S-KAIZEN:** Testing a new workplace organization method in one corner of a ward to improve efficiency before applying it to the entire department.
- **Infection Prevention & Control (IPC):** Piloting a new hand hygiene protocol during a single shift to identify challenges and measure compliance.
- **Safe Surgery:** Trialling the use of the Surgical Safety Checklist for five elective surgeries to identify and fix any workflow issues before making it a standard procedure.

Table 4.1 An example of Quality Improvement Plan (QIP) for Maternity & Neonatal Wards

Problem Identified	Intervention(s)	Actions & Resources Required	Timeframe	Responsible Person(s)	Evidence of Completion	Review Date	Outcome After Review
Inconsistent partograph completion in labour ward.	Strengthen partograph use and monitoring.	Conduct refresher training; develop a peer-review system; daily file audits by the in-charge. Resources: Partograph charts, audit checklist.	3 Months	Maternity In-charge, Senior Midwife	Completed audit forms; monthly report on completion rates.	Weekly	Improved partograph completion rate to >90%.
High rate of birth asphyxia.	Implement "Helping Babies Breathe" (HBB) drills.	Train all staff in HBB; conduct weekly mock drills; ensure daily checks of resuscitation equipment. Resources: HBB mannequins, equipment checklist.	6 Months	Neonatal Focal Person, Maternity In-charge	Training records; drill attendance sheets; equipment logs.	Monthly	Reduction in birth asphyxia rates by 50%

4.5 Monitoring, Sustaining, & Spreading Success

After a change has been successfully implemented, the final step is to monitor it to ensure it is sustained and delivering the desired results. Monitoring helps you track your performance over time and ensures that improvements become part of the new standard of care.

Key Monitoring Areas:

- **Client & Staff Satisfaction:** Use surveys, suggestion boxes, and interviews to regularly gather feedback. A happy and engaged workforce often leads to happier patients.
- **Turn-Around Time (TAT):** Measure the time it takes to complete key processes (e.g., from laboratory sample collection to result delivery). Reducing TAT is a key indicator of improved efficiency.
- **Respectful Compassionate Care (RCC):** Observe and get feedback on whether care is delivered with kindness, empathy, and dignity. The "6 Cs" (Commitment, Conscience, Competence, Compassion, Confidence, and Courage) are a great framework.

By continuously monitoring these areas, you complete the QI cycle and begin looking for the next opportunity for improvement, ensuring that quality becomes a core part of your facility's culture. In the next chapter, we will explore this step in greater detail.

CHAPTER 5: MONITORING, EVALUATION AND LEARNING

Monitoring and Evaluation (M&E) is the compass that tells us if our QI efforts are heading in the right direction. It is a continuous process that helps us answer one fundamental question: "Are the changes we are making actually leading to better care?" Monitoring involves the continuous tracking of our progress, much like checking the speedometer while driving, while evaluation is the periodic assessment of our results to see how far we've come. This chapter provides a clear framework to help your team build an effective M&E system, turning raw data into informed decisions and good intentions into measurable results. By integrating M&E into your QI work, you create a powerful feedback loop that drives a culture of learning and accountability.

5.1 Indicators

An effective M&E process begins with a clear and focused goal. Before you can measure anything, you must know what you are trying to accomplish, and these objectives should always be SMART: Specific, Measurable, Achievable, Relevant, and Time-bound. A well-defined goal, such as "reducing hospital-acquired infections by 20% within 12 months," provides the foundation for selecting the right indicators—the specific, measurable signs that will tell you if you are on track.

To get a complete picture of your performance, it is best to use a balanced set of indicators. These typically fall into three categories;

- **Structure indicators** measure the resources you have, such as the staff-to-patient ratio or the availability of essential equipment.
- **Process indicators** measure the actions you take, like the percentage of staff adhering to hand hygiene protocols.
- **Outcome indicators** measure the results you achieve, such as the rate of patient readmissions or scores from customer satisfaction surveys. By tracking a mix of these, you can understand not only your results but also the systems and processes that produce them.

Note that while national priorities (e.g., RMNCAH, IPC) are essential, your facility should also track indicators (Annex 1.2) that reflect its unique challenges and business health. Some of those useful indicators are listed below;

- Patient & Staff Experience: Staff Satisfaction Scores, Customer Satisfaction Scores.
- Efficiency: Average Turn-Around-Time (TAT) for lab results, Bed Occupancy Rate, Average Length of Stay.
- Financial Health: OPD visits vs. revenue, Rejection Rate for Insurance Claims.
- Clinical Outcomes: Morbidity and Mortality Rates, Surgical Site Infection (SSI) Rates, Neonatal Sepsis Rates.

5.2 Data collection

Your facility is already producing a wealth of data every single day. The key is knowing where to look. Your QIT should regularly collect information from a variety of sources to get a complete picture of the facility's performance.

Key data sources include:

- **Health Management Information System (HMIS):** This includes routine data from platforms like DHIS2, which tracks service delivery, disease trends, and key performance indicators.
- **Patient Registers (MTUHA) and Files (physical and electronic):** These contain detailed clinical information, patient demographics, and treatment outcomes.
- **Client Feedback Mechanisms:**
 - Suggestion Boxes: Offer raw, honest feedback directly from patients about their experiences.
 - Satisfaction Surveys & Exit Interviews: Provide structured feedback on specific aspects of care, like waiting times or staff attitudes.
- **Financial Records:** Data on revenue, expenditure, and insurance claims can highlight areas of inefficiency or growth.
- **Supervision and Audit Reports:** Checklists from Internal Supportive Supervision (ISS) or results from clinical audits provide expert assessment of compliance with standards.
- **Incident Reports:** Documentation of events like patient falls or medication errors is crucial for identifying and addressing safety risks.
- **Pharmacy and Laboratory Records:** Data on stock levels, drug dispensing, or test turnaround times can reveal bottlenecks in critical support services.

5.3 Data Analysis and Interpretation

Analysis is the process of turning that data into a story that reveals patterns and insights. A simple and powerful way to do this is to look for trends over time using a visualizations like Run Chart, which plots your performance monthly or weekly. This tool along with other charts is enlisted in the Annex. However, the charts you create are only as reliable as the data you put into them. If the initial data is wrong, your conclusions will be wrong too. This is the principle of "Garbage In, Garbage Out." To make good decisions, you must start with good data. To ensure quality data, focus on these four key areas:

- **Accuracy:** Is the data correct? A simple check like ensuring the number of patients in the register matches the number in the HMIS report can prevent major errors.
- **Completeness:** Is any information missing? An incomplete patient file or a register with empty columns can hide a serious problem.
- **Timeliness:** Is the data current? Using last year's data to solve today's problems is like driving while looking only in the rearview mirror. Data should be collected and analysed promptly.
- **Consistency:** Is the data recorded the same way every time? For example, everyone should use the same format for dates or the same codes for diagnoses to avoid confusion.

☒ Simple Data Quality Checklist:

- Does the data we collected make sense?
- Are there any obvious errors or empty fields?
- Did we count and record it correctly?
- Is it the most recent data available?

5.4 Reporting and Feedback Mechanisms

The insights gained from monitoring and evaluation are only valuable if they inspire action. Effective communication turns data into a powerful story that drives improvement. This session focuses on how to share M&E findings clearly, concisely, and in a way that is tailored to different audiences.

5.4.1 Core Principles of QI Communication

All reports and feedback, regardless of format, should be:

- **Clear & Concise:** Use simple language, charts, and graphs to highlight key findings. Avoid dense, lengthy reports.
- **Audience-Focused:** Tailor the information to the specific needs of the audience, whether it's frontline staff, department heads, or hospital leadership.
- **Action-Oriented:** Include not just data, but also success stories to build morale and provide clear, actionable recommendations for the next steps.

5.4.2 Key Mechanisms for Communicating M&E Findings

1. Regular Performance Reports. Disseminate reports to relevant stakeholders (e.g., QI committees, hospital leadership) on a predetermined schedule, such as monthly or quarterly. These reports should highlight key findings, track progress towards objectives, and identify areas requiring attention.

2. QI Dashboards. A modern and highly effective tool, **QI Dashboards** summarizes most important indicators. Dashboards provide real-time or near-real-time data, making it easy for leadership and staff to track performance at a glance.

3. Structured Feedback Loops. Establish formal mechanisms to provide direct feedback to frontline staff and teams on their performance. This closes the loop between data collection and on-the-ground practice. Effective methods include:

- Team meetings and huddles
- One-on-one discussions
- Educational sessions

4. Broader Dissemination for Transparency and Learning. Extend communication beyond your immediate facility to foster accountability and shared learning.

- **Public Reporting:** For certain indicators, publicly reporting quality data enhances transparency and accountability to the community you serve.
- **Learning Platforms:** Participate in or establish forums where different facilities can share best practices, challenges, and lessons learned from their QI initiatives.

5.4.3 Data Protection and Data Sharing

Quality Improvement relies on data-driven insights. If data is compromised or incomplete due to poor security practices, it undermines the integrity of QI processes. These are critical components of QI in health care settings, especially in faith-based facilities where trust and ethical standards are paramount.

5.5 Utilization of Findings

This is the final phase of the M&E cycle, where raw data is turned into tangible progress. Think of data as raw ingredients. By themselves, they have potential but offer little value. This utilization phase is the "cooking" as it's where insights from the data are used to create specific strategies for improvement. Without this step, M&E becomes an academic exercise where reports are filed away on a shelf, and the valuable lessons learned from the data are lost. Meaningful action means using the findings to make evidence-based decisions: celebrating and scaling up what works, redesigning what doesn't, and reallocating resources to areas of greatest need.

5.5.1 Learning

Learning is the critical step that transforms monitoring and evaluation into sustainable improvement. It involves reflecting on data, successes, and challenges to generate actionable insights that guide future practice. By documenting lessons, sharing experiences across teams, and fostering peer-to-peer learning, facilities create a culture of continuous improvement. This process ensures that knowledge gained from QI initiatives is not lost but translated into better strategies, policies, and daily practices. Embedding learning into QI cycles strengthens institutional memory, promotes innovation, and empowers staff and communities to adapt to emerging needs while sustaining long-term gains

5.5.2 Translating M&E Findings into Action

The findings from your M&E process should guide your team to celebrate successes, adapt improvement plans, and ensure that positive changes are sustained. Here's how to put your data to work:

1. Develop Specific Action Plans. Based on M&E results, create clear and targeted action plans. These plans should outline specific steps to address identified gaps or to sustain and spread positive changes.

2. Guide Resource Allocation. Use M&E findings to strategically allocate resources such as staff time, budget, and equipment to the area's most in need of improvement. This ensures you are investing in changes that will have the greatest impact.

3. Revise Policies and Procedures. Data may reveal that existing policies, clinical protocols, or standard operating procedures need to be updated. Use your findings as evidence to make necessary revisions that lock in improvements.

4. Identify Training and Education Needs. Performance gaps revealed by M&E often point to a need for further staff training. Use the data to identify specific knowledge or skill gaps and develop targeted educational sessions.

5. Review and Adapt the M&E Framework Itself. A good M&E framework is a living document. Periodically review and adapt it to ensure its continued relevance and effectiveness. This includes reassessing indicators, data collection methods, and reporting formats.

By systematically using M&E findings to inform these actions, your healthcare facility can ensure that QI initiatives lead to tangible and sustainable improvements in patient care and outcomes

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LIST OF ANNEXIES

Annex 1: Annual Quality Improvement Budget Guideline

BUDGET ITEM	FREQUENCY
QIT meeting with management	4 times (Every quarter)
QIT meetings sitting allowance	12 meetings
Internal Supportive Supervision	Monthly
Clinical Audit Allowance	4 times (Every quarter)
Stationeries	As per unit needs *
Customer Care Week	As per unit needs **
Blitz Exercises	4 times (Every quarter)
Experience sharing in every facility	Once per year (Optional)

NOTE: The stationeries need include but are not limited to the following: Office white papers, office-coloured papers, Ribbons, thumb stack pins, stapler and staples, punch machine, sole tape, coloured printer, toner etc.

Customer Care Week: Commemoration of the customer care week is an essential part of increasing awareness of the customer care culture among facility staff.

Annex 2: Quality Improvement Team Budget Template

SN	Activity	Description	Qty	Each	Total	Narrative
1	QIT Member	Responsibility Allowance	12	10,000	120,0000	The QIT is tasked with the duty of addressing all quality related matters in the facility
2						

Annex 3: Quality Improvement Team Members Appointment Letters

DATE:/...../.....

DEAR SIR/MADAM:

REF: APPOINTMENT TO BE A QUALITY IMPROVEMENT TEAM MEMBER

Refer to the heading above.

We are glad to inform you that you have been selected to join the quality improvement team as a member.

The following will be the activities you are expected to perform in this new task:

QIT MEMBER

1. Responsible for training of hospital staff
2. Conducting situation analysis before implementation of QI approach
3. Implementing QI activities for common problems of the hospital
4. Conducting periodical monitoring and provide technical advice to WITs
5. Conducting internal assessments for recognition against given standards with HMT
6. Responsible for recording of all QI activities conducted in the hospital
7. Reviewing situation and the action plan
8. Providing necessary input for QI activities
9. Producing Progress report bi-annually and share with WIT, HMT, and CHMT/RHMT/MOH according to the level of hospital

We are looking forward for your cordial cooperation.

.....

MEDICAL OFFICER INCHARGE

Annex 4: Quality Improvement Team Orientation Package

HEALTH FACILITY ORIENTATION PROGRAM SCHEDULE AND CHECKLIST

EMPLOYEE:

DESIGNATION:

DEPARTMENT:

DATE OF JOINING QIT:

SN	DEPARTMENT	PERSON	DATE	SIGN
	QIT IMPROVEMENT TEAM			
1	QIT Organizational Structure			
2	QIT Operational Plan			
3	QIT Sub-committees			
4	QIT Work Improvement Teams			
5	QIT Annual Plan			
6	QIT Key Performance Indicators			
7	QIT Meeting Schedule			
8	QIT Management Duties			
9	QIT Clinical Duties			
10	QIT Clinical Support Duties			
11	QIT Ancillary Services			
12	Clinical Audit			
13	Infection Prevention Control			
14	Waste Management			
15	Risk Management			
16	Internal Supportive Supervision			
17	Training Manual			
18	Occupational Health and Safety			
19	Fire Safety Training			
20	SOPs, Guidelines and Policies			
21	Quality Improvement Plan			
22	Quality Improvement Report			

EMPLOYEE SIGNATURE: DATE:

SUPERVISOR SIGNATURE: DATE:

Annex 5: Quality Improvement Team Report Template

The quality improvement team report template is as shown below:

- ✓ Heading of the report
- ✓ Table of contents
- ✓ Abbreviations
- ✓ Introduction
- ✓ Management Report
- ✓ Clinical Report
- ✓ Supportive Clinical Report
- ✓ Ancillary services Report
- ✓ IPC Status
- ✓ Internal Supportive Supervision report
- ✓ Customer care reports
- ✓ Clinical Audit reports
- ✓ KPIs Status
- ✓ Lesson Learnt
- ✓ Challenges
- ✓ Way forward

Tips of a good report:

A good report should contain pictures of different scenarios in the facility during your assessment of the facility.

A good report should contain figures, statistics and different data according to the needs.

A good report should contain visualization aids e.g. Bar charts, Line graphs, Scatter plots etc.

Annex 6: Quality Improvement Monthly Agenda

- Opening the meeting
- Report on implementation from the previous month
- Allocation of new responsibilities for the month
- Data/statistical report
- Any Other Business (AOB)

Suggested Additional Agenda Items

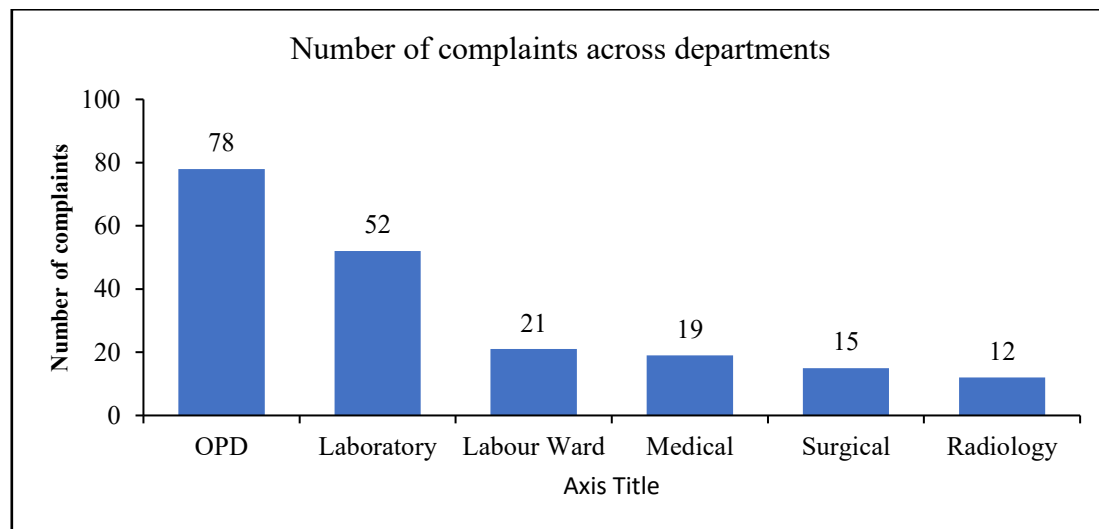
- Review of ongoing Quality Improvement (QI) projects and progress towards targets
- Discussion of challenges encountered and proposed solutions
- Updates on clinical audit findings and follow-up actions
- Training or refresher session on QI tools and methods (e.g., PDSA cycle, root cause analysis)
- Sharing best practices and success stories from departments/units
- Staff and patient feedback review
- Planning for upcoming QI activities, supervision, or mentorship sessions
- Resource and supplies updates to support QI initiatives
- Monitoring and evaluation alignment with facility or regional health priorities

Annex 7: Quality Improvement Team Action Plan Template

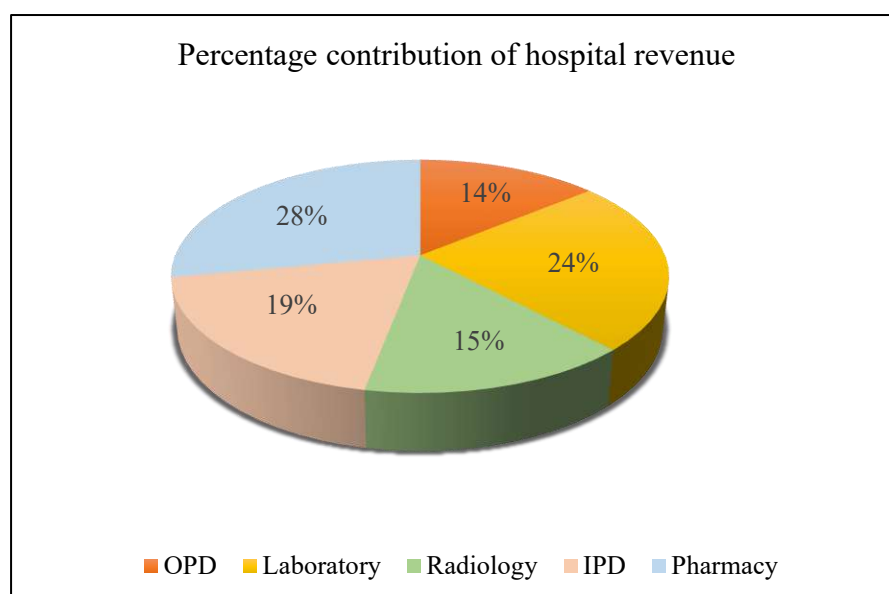
IDENTIFIED GAP	ACTION PLAN	RESPONSIBLE PERSON	TIME FRAME

Annex 8: Data visualization tools in QI activities.

- **Bar Chart** is useful for comparing different groups. For example, the number of patient complaints across various departments (e.g., OPD, IPD, Laboratory). Based on this chart, the QIT would be prompted to address complaints in the OPD and Laboratory departments, where the most significant issues appear to occur.

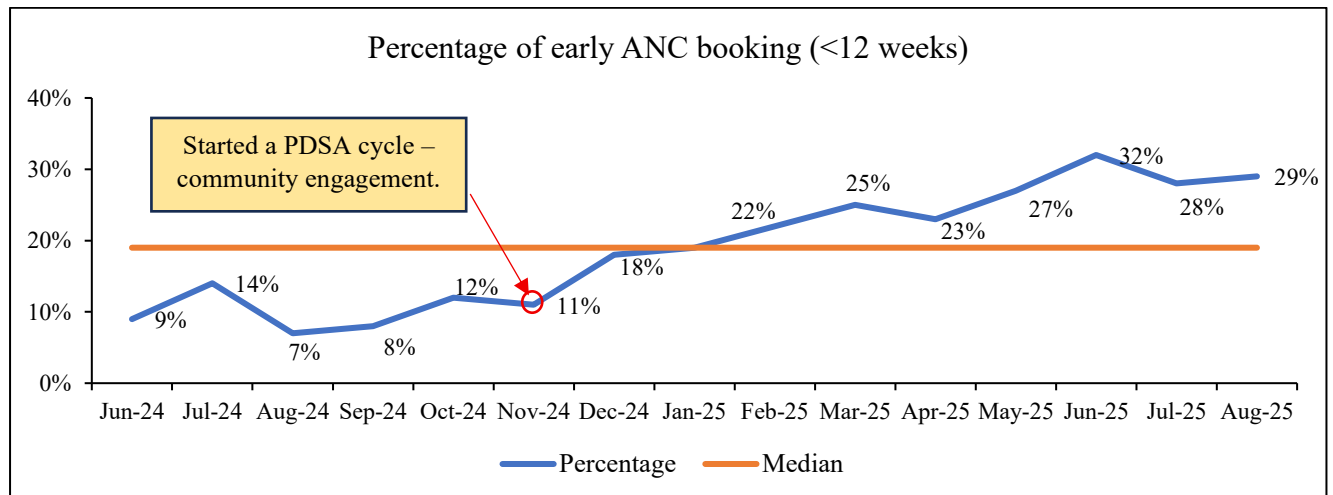


- **Pie Chart** can be an effective tool for illustrating how different departments contribute to the facility's total revenue. By visualizing the percentage share of each department, it becomes easier to identify which areas generate the most income. This insight can guide strategic planning to enhance revenue collection—especially by leveraging the potential of high-demand services.

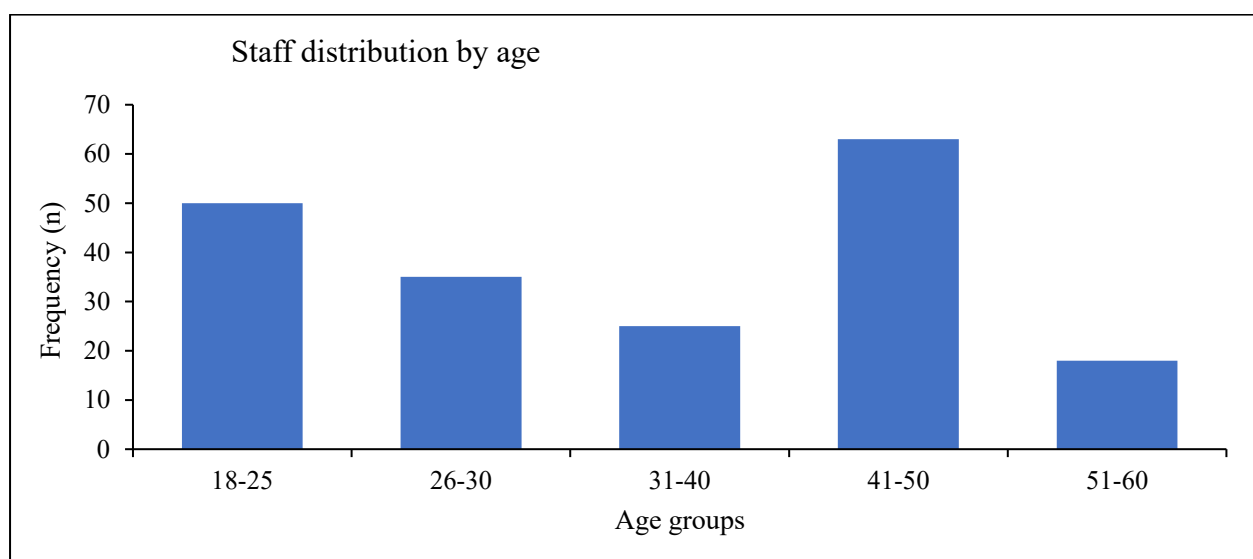


Tip: Avoid using pie charts for more than 5-6 categories, as they become hard to read.

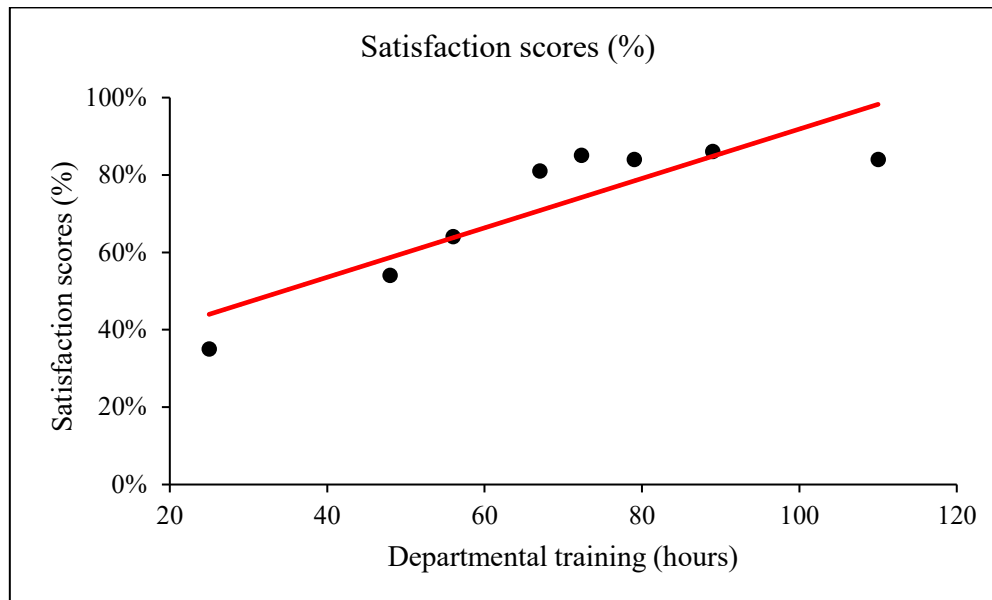
- **Line Graph (Run Chart).** This is perhaps the most important tool in QI. A run chart is a line graph that shows how a measure or indicator changes over time. It helps you see if the changes you are making are actually leading to an improvement. For example, you can track the monthly percentage of pregnant mothers who books early for antenatal care before and after you started a new community outreach program.



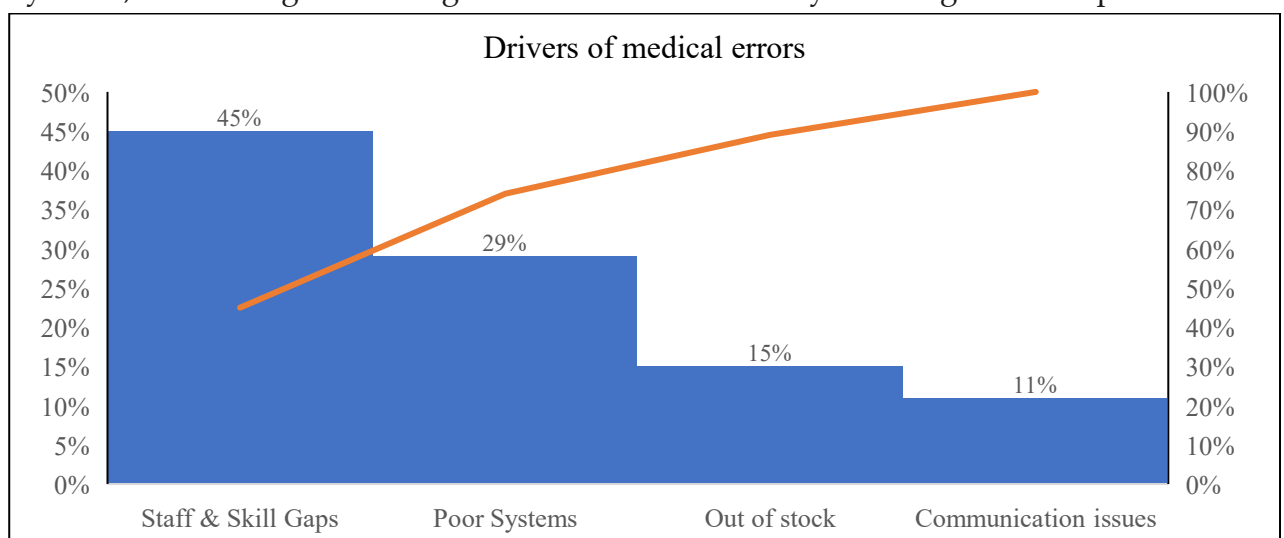
- **Histogram.** Although a histogram resembles a bar chart, it serves a distinct purpose: visualizing the distribution or frequency of data. We use a histogram to illustrate the age distribution of staff at the facility. This information is valuable for management when planning recruitment strategies, designing age-appropriate incentive packages, and forecasting future workforce needs. Histograms can also be useful for displaying patients waiting time thus helping to identify peak congestion periods and units experiencing extreme delays.



- **Scatter Plot** are useful for exploring relationships between two continuous or numerical variables. For example, you can plot training hours per department against monthly patient satisfaction scores. The Plot below shows a positive correlation—departments with more training hours tend to have higher satisfaction scores. This may suggest that regular strategic training helps staff identify service bottlenecks and implement improvements that enhance patient experience.



- **Pareto Chart:** This is a special type of chart that helps you prioritize problems. It's a combination of a bar chart and a line graph that follows the 80/20 rule (the pareto principle). The idea is roughly 80% of the problems come from 20% of the causes. From the chart, the leading contributors are “Staff & Skills Gaps” and “Poor Systems,” indicating where targeted interventions could yield the greatest impact.



Annex 9: SafeCare Levels



Annex 10: Algorithm for Assessment Scoring

SafeCare Rating and Advanced Assessment						
Legality Criterion (1.1.3.1) NC	Level 0					
Legality Criterion PC or FC	See level distribution below (at least level 1)					
# of Critical criteria at least PC Overall Score	0 to 9	10 to 14	15 to 19	20 to 24	25	OR from all applicable critical criteria NO NC allowed*
0 to 24	1	1	1	1	1	
25 to 39	1	2	2	2	2	
40 to 59	1	2	3	3	3	3
60 to 79	1	2	3	4	4	4
80 to 100	1	2	3	4	5	5

Annex 11 A. IPC (Infection Prevention and Control) Team

Members	Responsibilities (Key Focus: IPC)
Doctor, Nurse, Laboratory, & Pharmacy staff (Core clinical members)	Policy & Resources: ensure availability of IPC related SOPs and IPC equipment (e.g., color-coded dustbins).
	Education & Training: plan for facility staff education on IPC practices.
	Monitoring & Improvement: conduct regular IPC related supportive supervision and monitoring of IPC practices.
	Data & Accountability: display data related to IPC to show progress. Correcting the IPC gaps and implementing Quality Improvement plans (QIP) for IPC activities. Establish accountability tools/targets on IPC.
	Coordination: Presenting IPC issues in QIT and HMT meetings. Liaising with the QIT leadership to present facility requirements to improve IPC with the HMT.
	Recognition: Coordinate recognition of the best performing individuals/departments in IPC.

Annex 11 B. Clinical Services Team

Members	Responsibilities (Key Focus: quality of clinical services)
Doctor, Nurse, Laboratory, & Pharmacy staff (Core clinical members)	Audit & Action: Coordinate quarterly clinical audit and dissemination of findings in the health facility. Develop action plans and follow up of the implementation of the agreed action plans.
	Resources & Policy: ensure availability of guidelines, policies, and SOPs for Management of patients.
	Improvement: ensure implementation of gaps identified in clinical services in QIP.
	Preparedness & Accountability: facilitate training on emergency preparedness. Establish accountability tools/targets on clinical audits.
	Data & Coordination: displaying data to the health facilities to discuss progress. Liaising with the QIT leadership to present requirements to improve clinical outcomes/audits with the HMT.
	Recognition: Recognition of the best performing individuals/department in Clinical Audit.

Annex 11 C. Customer Care Team

Members	Responsibilities (Key Focus: client experience)
Nurse, Social worker, HR officer, IT, cashier	Feedback & Surveys: • Conduct quarterly Patient satisfaction surveys. • Conduct daily patients exit interviews. • Conduct bi-annual Staff satisfaction Surveys. • Coordinate the use and opening of suggestion box. • Conduct Community needs assessment.
	Complaints Management: establish a standard complaints procedure. manage patients' complaints and provide feedback to patients/staff regarding their complaints.
	Service Delivery: monitor the turnaround time of services offered and patients' flow. ensure availability of Triage.
	Environment & Personnel: ensure privacy is maintained at the hospital. ensure availability of a customer-services personnel ("niulize mimi").
	Training & Improvement: conduct regular continuous medical education on customer care. ensure implantation of qip activities related to the same.
	Coordination & Accountability: liase with QIT leadership to present the findings/requirements to the HMT. Establish accountability tools/targets on customer care.

Annex 11 D. Risk Management Team

Members	Responsibilities (Key Focus: Safety, Incidents, and Occupational Health)
Laboratory, Nurse, Doctor, Pharmacy & supporting staff e.g., cleanliness staff, watchman, etc	Policy & Training: develop policies and ensure availability of SOPs in relation to risk. Conduct regular education on risk related policies.
	Assessment & Incident Management: conduct risk assessment. recording of incidents in an incident book and addressing issues accordingly during QI-Meetings by developing action plans.
	Occupational Health & Safety: establish an occupational health program for staff, including staff vaccinations and fire safety programs. Ensure availability of a staff vaccination program.
	Improvement: ensure implementation of the gaps identified in the Quality Improvement plans.

Annex 11 E. Maintenance and Hygiene Team

Members	Responsibilities (Key Focus: Facility, Equipment, and Support Services)
Laboratory, Nurse, Doctor, Pharmacy & supporting staff e.g., cleanliness staff, watchman, etc	Maintenance & Assets: ensure availability of a fixed assets register. Ensure regular facility maintenance. Ensure availability of a planned maintenance schedule for all biomedical equipment.
	Hygiene & Housekeeping: follow-up on cleaning schedules. Coordinate continuous medical education schedules on Hygiene and cleanliness.
	Support Services: coordination of food services and Coordination of laundry services.
	Improvement: ensure implementation and maintenance of gaps identified in the QIP.

Annex 12: Annual QI- Plan

S/n	Activities	Target	Year.....											
			J	F	M	A	M	J	J	A	S	O	N	D
1.	Quality improvement meetings	Monthly												
2.	IPC monitoring	Monthly												
3.	Clinical audit	Quarterly.												
4.	Patient exit interviews	Daily												
5.	Staff satisfaction survey	Bi-annually												
6.	Quality improvement team and management meeting	Quarterly.												
7.	Opening of suggestion boxes	Bi-monthly or weekly depending with patient volume.												
8.	Patient satisfaction survey	Quarterly												
9.	Continuous medical examination	Weekly at specific departments with WITs.												
10.	Hospital inspection	Quarterly												
11.	Emergency training	Quarterly												
12.	Staff immunization	Bi-annually.												
13.	Fire safety training	Annually.												
14.	Conduct risk assessment	Bi-annually												
15.	Quarterly meetings to discuss top 10 diseases and hospital data	Quarterly												

*This QI plan provides an example of QI activities that can be implemented in a particular year. Facility QITs may add or omit other activities as contextual needs necessitates.

Annex 13 A: Customer Care Indicators

Indicator	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Staff Satisfaction Rate	The proportion of staff who report being satisfied with their job, working conditions, and organizational support.	80%	Number of staff reporting "satisfied (scored 80%+)	Number of staff surveyed	HR & QI office	Biannually	HR & QI focal
Exit Interview Completion Rate	The proportion of departing staff who completed an exit interview before leaving the organization.	100%	Number of staff who completed an exit interview	Total number of staff who exited	HR & QI office	Annually	HR & QI focal
Client Exit Interview Completion Rate	The proportion of clients who completed an exit interview after receiving services at the hospital.	100%	Number of clients who completed exit interview	Total number of clients had an exit interview	Customer Care file	Monthly	Customer care focal
Clients' satisfaction rate	The proportion of clients who report being satisfied or very satisfied with the services they received at the facility,	80%	Total number of clients surveyed and Number of clients reporting "satisfied	Number of staff surveyed	Customer Care file	Monthly	Customer care focal
Client Complaints Resolution Rate	The proportion of client complaints received that were resolved within the reporting period.	100%	Number of complaints resolved	Total number of complaints received	Customer Care file	Monthly	Customer care focal
Client Privacy Satisfaction Rate	The proportion of reported client privacy issues/complaints that were resolved within the reporting period.	100%	Number of privacy issues resolved	Total number of privacy issues reported	Customer Care file	Monthly	Customer care focal
Privacy Issue Resolution Rate	The proportion of issues raised through community-led monitoring mechanisms that were resolved within the reporting period.	100%	Number of community issues resolved	Total number of community issues reported	Customer Care file	Biannually	Customer care focal

Annex 13 B: Service Efficiency Indicators

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Average client time spent facility	The average time clients spend waiting from the moment they arrive at the hospital/service point until they are attended at the dispensing window	Normal Track (Cash & Insurance): 120 minutes; Fast Track (Cash and Insurance): 45 minutes	Total time spent by all clients at the facility	Total number of clients observed	Hospital EMR system	Weekly	Customer Care focal
Average Discharge Waiting Time per Department	The average time clients spend waiting to be discharged in each hospital department.	180 minutes	Total discharge waiting time for all clients in a department	Total number of clients discharged in that department	Hospital EMR system	Weekly	Customer Care focal
Surgery Postponement Rate	The proportion of scheduled surgeries that were postponed to a later date within a given reporting period.	0%	Total number of surgeries which were postponed	Total number of surgeries scheduled	Hospital EMR system	Weekly	Customer care focal

Annex 13 C: Training Indicators

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Proportion of staff trained in QI	The proportion of facility staff who have received formal training in Quality Improvement (QI) methods within a specified period.	100%	No. of staff trained in each department	Expected number of staff to be trained	QI File	Quarterly	QI Focal
Proportion of Staff Attending CME Sessions	The proportion of staff who have participated in at least one Continuous Medical Education (CME) session within a specified reporting period.	100%	Number of staff attending CME sessions	Total number of staff	CME file/Book	Weekly	Training coordinator
Proportion of Staff Trained in Emergency Care	The proportion of facility staff who have received formal training in emergency care (including emergency response, life support, trauma management) within a specified reporting period.	100%	Number of staff trained in emergency care	Expected number of staff to be trained	Emergency file	Quarterly	QI Focal
Proportion of Staff received an Orientation & Induction Training	The proportion of newly recruited staff who have received orientation and induction training within a specified period.	100%	Total number of new staffs oriented	Total number of new staffs	QI, Matron, HR and Clinical coordinator files	Quarterly	QI focal, Matron, HR and Clinical coordinator
Proportion of Staff Trained in Fire Safety	The proportion of facility staff who have received formal fire safety training within a specified period.	100%	Total number of staffs trained	Total number of targeted staff to be trained	Risk Management file	Biannually	Risk manager

Annex 13 D: Immunization Indicators

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Proportion of Staff Immunized	The percentage of facility staff who have received all required immunizations according to national guidelines or facility policy.	100%	Number of staff who received recommended immunizations	Total number of staffs in the facility	OHS file	Quarterly	OHS focal
Facility Risk Status	The proportion of health facilities assessed that fall into each risk category (low, moderate, or high) based on compliance with safety, infection prevention, and preparedness standards.	0%	Total number of risks addressed	Total number of risks identified	Risk Management file	Semi Annual and Annual	Risk focal
Proportion of Facilities with Adequate Waste Management Practices	Key Standards to Assess (examples): availability of color-coded bins/containers for waste segregation, proper segregation at the point of generation, availability and use of PPE, etc in compliance with national or WHO waste management guidelines.	100%	Number of facilities with adequate waste management practices	Total number of facilities assessed	Waste file	Quarterly, annually	Waste focal

Annex 13 E: Quality Improvement Indicators

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Proportion of Planned QI Meetings Conducted	The percentage of scheduled Quality Improvement (QI) meetings conducted during the reporting period.	12	Total QI meetings conducted	Total no. of QI meetings planned in one year	QI file	Quarterly, annually	QI focal
Proportion of QI projects implemented as planned	Measures the percentage execution of the approved QI projects within a given reporting period.	100%	Number of QIPs implemented as planned	Total number of QIP approved	QPP & QI files	Quarterly, annually	QI focal
Proportion of QI and management meetings conducted	The percentage of QI and Management meetings that were actually conducted within a given reporting period.	4	Total number of meetings conducted between QI & management	Total number of meetings planned in one year	QI file	Quarterly, annually	QI Focal
Proportion of Clinical Audits Conducted as Planned	Measures the percentage of scheduled clinical audits that were actually conducted within a reporting period.	4	Number of Clinical Audit conducted	Number of Clinical Audit conducted	Clinical Audit file	Quarterly, annually	Clinical Audit focal
Proportion of IPC gaps addressed	Measures the percentage of identified infection prevention and control (IPC) gaps that have been corrected or mitigated within a specific reporting period.	100%	Total number of IPC gaps addressed	Total number of IPC gaps identified	IPC file	Quarterly, annually	IPC file
Internal Supportive Supervision	Measures the percentage of scheduled internal supportive supervision visits that were actually carried out within a facility during a reporting period.	100%	Total number of gaps addressed from ISS reports	Total number of gaps identified	ISS file	Quarterly, annually	ISS focal

Annex 13 F: Quality of Service

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Readmission Rate within 7 Days of Discharge	The proportion of patients who are readmitted to the same facility (or any facility, depending on scope) within 7 days of being discharged from an inpatient department.	0%	Total number of readmitted patients in one week following their discharge	Total number of discharged patients	Medical Records, EMR	Monthly /Quarterly	Medical Records focal
Reoperation Rate among Post-Surgical Patients	The proportion of patients who require a reopening (reoperation) after undergoing a surgical procedure within a specified period (e.g., during the same admission or within 30 days of the initial surgery).	0%	Number of patients who required surgical reopening after an initial operation (due to complications such as bleeding, infection, or failed procedure).	Total number of patients who underwent surgery during the reporting period.	Surgical Department	Monthly /Quarterly	Surgical focal

Annex 13 G: Service Safety

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Surgical Site Infection Rate	The proportion of patients who develop a surgical site infection (SSI) within 30 days after surgery (or within 90 days if a prosthesis is implanted).	0%	Number of surgical patients who developed SSI within the specified period	Total number of patients who underwent surgery	QI file, Patient record, Operating theatre register	Monthly	QI Focal
Phlebitis Rate	The proportion of patients with peripheral intravenous (IV) catheters who develop phlebitis during their hospital stay.	<5%	Total number of clients who develop phlebitis	Total number of cannulated patients		Monthly	QI Focal
Pressure Sore rate	The proportion of admitted patients who develop a new pressure ulcer (sore of any stage) during hospitalization.	0%	Total number of patients developing pressure sores	Total number of admitted patients in the ward	Customer care file	Monthly	Customer Care focal/ Matron
Incident Reporting Rate	The proportion of reported incidents out of the total number of patient encounters or staff activities, depending on context.	0%	Number of reported incidents in a period	Total number of patient encounters or staff activities	Risk Management file	Monthly	Risk Manager
Adverse Drug Reaction rate	The proportion of patients who experience one or more adverse drug reactions	0%	Number of patients with reported ADRs	Total number of patients who received medications	Risk file & Pharmacy file	Monthly	Risk Manager & Pharmacist
Patient Fall rate	The number of patients falls in a healthcare facility during the same period.	0%	Total number of patient falls	Total number of patients served in that particular ward	Risk file	Monthly	Risk Manager and WIT

Annex 13 H: Business Indicator

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Bed Occupancy Rate (BOR)	The proportion of available hospital beds that are occupied by patients over a specific period. It measures hospital capacity and/or utilization.	> 80%	Total inpatient days during the period	Total available bed days during the period	HMIS Register, EMR	Daily, Weekly, Monthly, or Quarterly	Data officer
Number of New Patients Attending Hospital	The total number of patients visiting the hospital for the first time (not previously registered in the hospital records) within a specified period.	100%	Total number of new patients attending at the hospital	Total number of patients attending	HMIS Register, EMR	Daily, Weekly, Monthly, or Quarterly	Data officer
Total Number of Patients Who Received Laboratory and pharmacy Services	The total count of patients who accessed and received any laboratory test(s) / received medication at pharmacy within the reporting period.	100%	Total number of patients receiving consultation, laboratory/ pharmacy services	Total number of patients attending at the hospital	HMIS Register, EMR	Daily, Weekly, Monthly, or Quarterly	Data officer
Total number of major surgeries performed	The total count of major surgical procedures carried out in the operating theatre during the reporting period.	100%	Total number of major surgeries performed	Total number of surgeries performed	Surgical Ward	Daily, Weekly, Monthly, or Quarterly	Surgical Focal

Annex 13 I: Security Indicators

Indicator Name	Definition	Target	Numerator	Denominator	Data Source	Frequency	Focal
Proportion of EHR Records Audited	The percentage of electronic health records reviewed (audited) for accuracy, completeness, and compliance with documentation standards.	100%	Number of EHR records audited	Total number of EHR records in the period	EHR	Monthly	IT consultant/QIT
Camera Monitoring Performance for Fraud Risk Mitigation	Measures the proportion of functional and actively monitored cameras in high-risk areas (e.g., pharmacy, stores, cash offices, laboratories, theatres) as a safeguard against fraud, theft, and malpractice.	100%	Total number of cameras required in high-risk areas	Number of functional cameras in high-risk areas	Security and surveillance reports, Fraud/risk management audits.	Monthly	IT/Security unit
Proportion of Drugs Audited	Measures the percentage of drugs in stock that are audited (checked for availability, accuracy of records, and compliance with storage/use standards) within a given reporting period.	100%	Number of Drugs audited	Total number of Drugs in stock	Pharmacy Department	Monthly	IT/Pharmacy staff